



GRANT HUANG, Ph.D.
ACTING CHIEF RESEARCH AND DEVELOPMENT OFFICER
OFFICE OF RESEARCH AND DEVELOPMENT (ORD)
BEFORE THE
HOUSE VETERANS AFFAIRS COMMITTEE SUBCOMMITTEE ON HEALTH

November 18, 2025

TABLE OF CONTENTS

TOPICS	TAB (hyperlink)
HVAC Health Subcommittee Members	1
ORD By The Numbers & HVAC Talking Points	2
Talking Points:	
• 208 (Conflict of Interest)	3
• Alternative Therapies – Mental Health	4 5
• Artificial Intelligence (AI)	6
• Clinical Trials	7
• Data and Safeguards	8
• Harvard Research	9
• HBOT (Hyperbaric Therapy)	10
• Mental Health	11
• Military Exposure	12
• MVP	13
• Precision Oncology	14
• Prosthetics	15
• Psychedelics	16
• Research Workforce (Field)	17
• Rural Health	18
• Sensitive Species	19
• Suicide Prevention and PTSD	20
• TBI and Blast Injuries	21
• Veterans Engagement	22
• Visual Health	23
• Women’s Health	

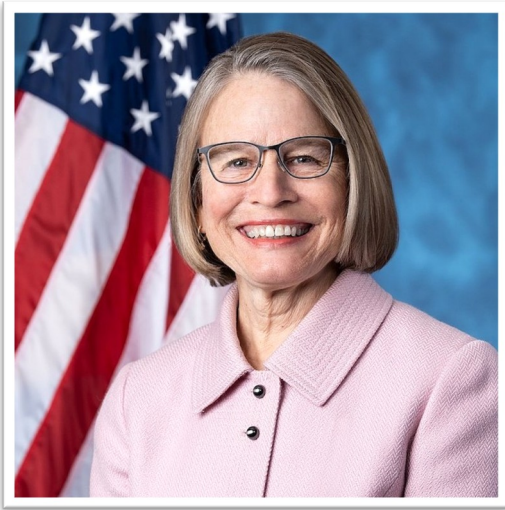


TAB 1

HVAC Health Subcommittee Members

HVAC Committee Members

Subcommittee on Health



Chairwoman:

Mariannette Miller-Meeks (R-IA)

Iowa's 1st District

Areas of Interest

- Health care, border security, education, veterans' services

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs (Chair, Subcommittee on Health)
- House Committee on Energy and Commerce

Research Projects

- Iowa City VA Health Care System
 - ORD – 44 (\$5 million)
 - Non-VA – 99 (\$2 million)

Additional Information

- U.S. Army veteran and **ophthalmologist** who served 24 years in the military.
- Focuses on ensuring equitable care for rural and women Veterans.

Sponsored / Supported Legislation

- Veterans CARE Act (2023): Directs VA to research medical cannabis as a treatment for PTSD and chronic pain.
- No Wrong Door for Veterans Act (2025): Modernizes VA suicide-prevention programs and coordinates crisis services across agencies.



HVAC Committee Members

Subcommittee on Health

Jack Bergman (R-MI)

Michigan's 1st District



Areas of Interest

- Military service, Great Lakes protection, rural health care

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs
- House Committee on Armed Services

Research Projects

- Oscar G. Johnson VA Medical Center (Iron Mountain) and Saginaw VA Medical Center
 - (No Research Projects)

Additional Information

- Retired U.S. Marine Corps Lieutenant General; senior-most combat Veteran in Congress.

Sponsored / Supported Legislation

- SAFE STEPS for Veterans Act: Creates a VA Office of Falls Prevention.
- RELIEVE Act: Expands emergency-care reimbursement for newly enrolled Veterans.

HVAC Committee Members

Subcommittee on Health



Greg Murphy (R-NC)

North Carolina's 3rd District

Areas of Interest

- Health care reform, veterans' mental health, medical research

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs
- Republican Doctors Caucus

Research Projects

- Durham VA Health Care System
 - ORD – 177 (\$18 million)
 - Non-VA – 383 (\$1.2 million)

Additional Information

- **Practicing urologist (MD)** and former medical-school faculty member.
- Advocate for medical innovation and VA workforce development.

Sponsored / Supported Legislation

- Veterans CARE Act (2023): Directs VA to research medical cannabis as a treatment for PTSD and chronic pain.
- No Wrong Door for Veterans Act (2025): Modernizes VA suicide-prevention programs and coordinates crisis services across agencies.



HVAC Committee Members

Subcommittee on Health

Derrick Van Orden (R-WI)

Wisconsin's 3rd District



Areas of Interest

- Agriculture, mental health, border security

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs
- House Committee on Transportation and Infrastructure

Research Projects

- Tomah VA Medical Center
 - (No Research Projects)

Additional Information

- Retired U.S. Navy SEAL; advocate for mental-health and transition programs

Sponsored / Supported Legislation

- VA Home Loan Program Reform Act (2025): Creates a permanent program preventing foreclosures.
- NOPAIN for Veterans Act: Expands access to non-opioid pain treatment.

HVAC Committee Members

Subcommittee on Health



Jen Kiggans (R-VA)

Virginia's 2nd District

Areas of Interest

- Defense, veterans' transition, health-care modernization

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs
- House Committee on Armed Services

Research Projects

- Hampton VA Medical Center
 - ORD – 2 (\$300K)
 - Non-VA – 10 (\$400K)

Additional Information

- **Nurse practitioner** and former U.S. Navy helicopter pilot.
- Focused on improving women Veterans' health and transition support.

Sponsored / Supported Legislation

- VA Hospital Inventory Management Act (2025): Modernizes VA supply-chain tracking.
- COPE Act: Expands mental-health grants for Veteran caregivers.



HVAC Committee Members

Subcommittee on Health

Abe Hamadeh (R-AZ)

Arizona's 8th District



Areas of Interest

- Border security, veterans' services, public safety

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs
- House Committee on Oversight and Accountability

Research Projects

- Phoenix VA Health Care System
 - ORD – 30 (\$2 million)
 - Non-VA – 102 (\$2 million)

Additional Information

- U.S. Army Reserve intelligence officer and attorney.

Sponsored / Supported Legislation

- HPSP Improvement Act (2025): Enhances VA scholarships for medical students and STEM fields.
- VET Access Act: Expands community-care eligibility in Arizona.

HVAC Committee Members

Subcommittee on Health



Delegate

Kimberlyn King-Hinds
(R-Northern Mariana Islands)

Northern Mariana Islands

Areas of Interest

- Island infrastructure, equitable Veteran benefits, health access

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs

Research Projects

- VA Pacific Islands Health Care System
 - ORD – 4 (\$500K)
 - Non-VA – 13 (\$0)

Additional Information

- Attorney and advocate for Pacific Island Veterans.

Sponsored / Supported Legislation

- TRAVEL Act (2025): Improves access to specialized VA care for territorial Veterans
- HOME Act (2025): Expands Veteran homeownership programs in territories



HVAC Committee Members

Subcommittee on Health

Ranking Member

Julia Brownley (D-CA)

California's 26th District



Areas of Interest

- Veterans' health care, women's equity, mental-health services

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs (Ranking Member, Health)
- Women Veterans Caucus (Co-Chair)

Research Projects

- VA Greater Los Angeles Health Care System
 - ORD – 156 (\$16 million)
 - Non-VA – 495 (\$10 million)

Additional Information

- Longtime advocate for women Veterans; focused on access to care and leadership

Sponsored / Supported Legislation

- Women Veterans Health Package (2025): Expands infertility and menopause services.
- Reproductive Freedom for Veterans Act: Protects reproductive health care at VA.

HVAC Committee Members

Subcommittee on Health



Sheila Cherfilus-McCormick (D-FL)

Florida's 20th District

Areas of Interest

- Health equity, economic mobility, housing security

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs
- Congressional Black Caucus

Research Projects

- West Palm Beach VA Medical Center
 - ORD – 1 (\$158K)
 - Non-VA – 1 (\$14K)

Additional Information

- **Health-care executive** focused on mental health and affordable housing for Veterans.

Sponsored / Supported Legislation

- VR&E Improvements Act: Strengthens Veteran Readiness & Employment services.
- PACT Act Expansion Initiative (2025): Adds glioblastoma as a presumptive condition.



HVAC Committee Members

Subcommittee on Health

Maxine Dexter (D-OR)

Oregon's 3rd District



Areas of Interest

- Public health, environmental protection, housing security

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs
- House Committee on Energy and Commerce

Research Projects

- VA Portland Health Care System
 - ORD – 143 (\$24 million)
 - Non-VA – 547 (\$20 million)

Additional Information

- **Pulmonologist (MD)**
- Advocate for mental-health and housing programs for Veterans.

Sponsored / Supported Legislation

- Every Veteran Housed Act (2025): Expands partnerships to eliminate Veteran homelessness.
- Veterans' Reproductive Care Protection Act (2025): Safeguards reproductive and maternity care.

HVAC Committee Members

Subcommittee on Health



Herb Conaway (D-NJ)

New Jersey's 3rd District

Areas of Interest

- Health care, medical ethics, Veterans' benefits

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs
- Doctors Caucus

Research Projects

- VA New Jersey Health Care System (East Orange Campus)
 - ORD – 20 (\$3 million)
 - Non-VA – 53 (\$371K)

Additional Information

- **Physician (MD, JD)**, former U.S. Air Force Captain.
- Focuses on medical staffing and research capacity at VA.

Sponsored / Supported Legislation

- TBI Research Amendment (2025): Expands VA TBI research oversight.
- Access to Care Improvement Amendment: Streamlines community-care referrals.



HVAC Committee Members

Subcommittee on Health

Kelly Morrison (D-MN)

Minnesota's 3rd District



Areas of Interest

- Health equity, reproductive rights, Veteran family support

Relevant Committees and Caucuses

- House Committee on Veterans' Affairs
- Democratic Women's Caucus

Research Projects

- Minneapolis VA Health Care System
 - ORD – 159 (\$14 million)
 - Non-VA – 517 (\$23 million)

Additional Information

- **OB/GYN physician (MD)**
- Advocate for women's and family health initiatives in VA policy.

Sponsored / Supported Legislation

- Justice for America's Veterans and Survivors Act (2025): Improves mental-health and suicide-prevention programs.
- Love Lives On Act (2025): Allows surviving spouses to retain benefits after remarriage.

HVAC Committee Members

Subcommittee on Health



Research Numbers

	VA - ORD		Non VA	
	PROJECTS	FUNDING	PROJECTS	FUNDING
Maxine Dexter, D-OR-03	143	\$ 24,163,278.00	547	\$ 19,651,044.00
Greg Murphy R-NC-03	177	\$ 18,068,061.39	383	\$ 1,246,386.95
Julia Brownley, D-CA-26	156	\$ 16,401,702.00	495	\$ 9,830,097.63
Kelly Morrison, D-MN-03	159	\$ 14,421,710.00	517	\$ 23,003,497.82
Mariannette Miller-Meeks, R-IA-01	44	\$ 4,809,217.00	99	\$ 2,465,004.30
Herb Conaway, D-NJ-03	20	\$ 2,619,071.47	53	\$ 371,963.61
Abe Hamadeh, R-AZ-08	30	\$ 2,292,708.36	102	\$ 1,815,958.00
Kimberlyn King-Hinds, R-Northern Mariana Islands	4	\$ 488,802.00	13	\$ -
Jen Kiggans, R-VA-02	2	\$ 264,658.00	10	\$ -
Sheila Cherfilus-McCormick, D-FL-20	1	\$ 158,624.00	1	\$ 14,417.00
Derrick Van Orden, R-WI-03				
Jack Bergman, R-MI-01				



TAB 2

ORD By The Numbers and HVAC Talking Points

HVAC Subcommittee on Health – Talking Points

ORD OVERVIEW – BASIC STATISTICS

	Year	
Number of ORD-Funded Researchers	FY24	3,774
Number of VA Researchers	FY25	~7,000
• NTEs		3,827
• IPAs		3,813
• WOCs		10,867
• Number of Research Employees on Term Appointments	FY25	19,061
Number of VA Research Sites	FY24	106
Number of VA Research Sites funded by ORD	FY24	102
Number of Publications	FY25	11,107
Number of ORD-funded Research Projects	FY24	7,431
Number of Research Projects (ORD or non-ORD funded)		
MVP Enrollment	FY25	1,090,780
Number of Active Clinical Trials (ORD and non-ORD)	FY25	2000+
• Number of Veterans enrolled in these Trials	Estimate	1,000,000+
Number of ORD-Funded Clinical Trials	FY25	462
• Number of Veterans enrolled in ORD Trials	Estimate	10,000+
• ORD funding for Clinical Trials	FY25	\$ 143.8M
Merit Review Applications Reviewed	FY25	1,913
Merit Review Applications Funded (Based on Spring and Winter applications = 972. Funded in Spring=102, Winter=70, for 18% total funded).	FY25	18%

VA Research Centennial Anniversary HVAC Talking Points

2025 marks the 100th anniversary of VA Research and honors a legacy of groundbreaking innovations and discovery that has transformed Veteran' health care. VA Research spans a range of activities that include understanding mechanisms of disease to studying new treatments and approaches to deliver care across a range of diseases and conditions that affect Veterans. For a century, VA Research has been instrumental in some of the most significant breakthroughs in medical history. Some select examples are below.

Key Achievements and Milestones:

- In 1925, the Medical Council of the Veterans Bureau began efforts to establish a research program to improve the quality of the professional staff and medical services for Veterans.
- VA's first research laboratory was the Tumor Research Laboratory at the Hines VA Medical Center, Chicago, Illinois, in 1933. While there, **Dr. Robert Schrek** first identified both the link between smoking and lung cancer and the link between sun exposure and skin cancer.
- Three VA researchers have won the Nobel Prize in Physiology or Medicine: **Dr. Rosalyn Yalow**, who uncovered the mechanisms of insulin injections in the body; **Dr. Andrew Schally**, who discovered how the hypothalamus links the nervous system to the endocrine system; and **Dr. Ferid Murad**, who discovered nitric oxide helps maintain healthy blood vessels.
- **Dr. Michael E. DeBakey** is often referred to as the father of modern cardiovascular surgery, with his VA research being instrumental to the heart-lung machine, coronary artery bypass, artificial heart, and ventricular assist pump. He also performed some of the first successful heart transplants, and created a synthetic material called Dacron to act as a prosthetic graft for weakened arteries while at the Houston VAMC that now bears his name.
- In 1961 VA Scientist **Dr. William Henry Oldendorf** received the patent for a device that could scan interior objects obscured by dense material, a prototype that led to the CT scanners and MRIs of today. His research for VA not only revolutionized medical care, but also the fields of security, archaeology, engineering, and much more.

- You've probably heard the phrase, 'Take two aspirin and call your doctor in the morning.' That practice began with a VA cooperative study, published in 1983, that proved aspirin could reduce the risk of heart attacks and strokes, and that high blood pressure was a treatable condition, saving a countless number of lives.
- **Dr. Jed Rose** and his fellow VA researchers developed the nicotine patch in 1984, famously first testing the patch on themselves to prove it was safe for Veteran volunteers.
- Findings from a clinical trial of over 38,500 individuals published in 2005 led by **Dr. Michael Oxman** and colleagues showed that a "zoster vaccine" was able to reduce pain from and prevent the occurrence of shingles, which comes from the chicken pox virus. These results led to the FDA approval of the vaccine for this extremely painful condition that affects many older adults.

2. Impact on Veteran Health:

- VA Research addresses the unique health challenges associated with Veterans' military service, resulting in new medical approaches to post-traumatic stress disorder (PTSD), traumatic brain injury (TBI), suicide prevention, and so much more. For example:
 - A 2021 VA study proved magnetic fields could be used to noninvasively stimulate electrical signals in the brain and reduce PTSD symptoms.
 - The Long-term Impact of Military-Relevant Brain Injury Consortium (LIMBIC) studies the effects of mild TBI on a cohort of over 3,100 Veterans, and the Total Brain Diagnostics initiative uses their findings to develop next generation diagnostics for TBI and other brain health conditions.
 - In 2024 VA facilities began offering firearm lockboxes to Veterans after VA Research showed increasing the time to access lethal means can reduce the number of suicides.
- VA's Military Exposures Research Program is shaping how toxic exposures are defined, identified, and treated, in accordance with the PACT Act. Such efforts include the Ranch Hand Study which linked several health conditions to Agent Orange exposure in 2006. Studies in 2013 and 2018 also linked it to heart disease and Parkinson's disease. Additionally, the Toxic Embedded Fragment Study Center studied the impact of embedded shrapnel containing lead, cobalt, or depleted uranium on Veterans. All this research continues to be examined as part of a VA co-led interagency work group to look at how toxic exposures can be studied and inform clinical practices for monitoring and treating affected Veterans.
- In 2014, the FDA approved VA's design for a powered arm prosthetic capable of multiple simultaneous movements, and in 2017 two Veterans were the first to ever receive this electromechanical limb. VA research continues to build off this achievement, with recent studies developing prosthetics that can be controlled by thought, restore a sense of feeling or balance, and be connected directly to the bone.
- VA Research and its partners from the VA National Oncology Program have established a new approach to integrate research with clinical care that includes the Lung Cancer Precision Oncology Program. With a particular focus on lung cancer screening, over 846,000 screenings among 388,000 Veterans have been completed over the past 4 years, an increase of 150% percent among eligible Veterans. More importantly, this screening has

shifted the stage of lung cancer diagnosis earlier where it can be more treatable and less deadly.

- VA Research unlocks new insights into how genetics, lifestyle, and military experiences impact health, paving the way for personalized medicine: tailor-made cures as unique as the Service member is. Recent examples include:
 - Identifying 12 gene variants that increase the risk of suicide in 2023.
 - Showing how genetic testing can avoid adverse drug interactions in 2022.
 - Finding 178 locations on the human genome that affect depression in 2021.

3. Future Directions:

- VA was home to the first clinical trial in the U.S., and it remains the #1 priority of ORD to increase Veterans' access to high-quality clinical trials as it allows volunteer Veterans to try tomorrow's treatments and therapies today. The feedback provided by these Veterans demonstrates the effectiveness of novel medical techniques in a variety of different situations, informing clinicians and policy makers alike in the use of cutting-edge health care and putting the VA Secretary's Veterans First priority into action.
- ORD's Cooperative Studies Program (CSP) brings together clinician-investigators, expert statisticians and a wide variety of specialists from 11 centers to plan and conduct large, multicenter clinical and epidemiological studies. One current trial has 50,000 participants nationwide to compare non-invasive screening with traditional colonoscopies, which is just one example of how CSP can expand the reach and number of VA's clinical trials into the future. CSP is also creating opportunities for trials to reach rural Veterans to give them access to these studies.
- With over a million Veterans enrolled, ORD's Million Veteran Program (MVP) is already the world's largest and most diverse genetic research database within a health care system. Contributing to over 120 research projects, over 450 health science publications, and over 200 million analyses, MVP continues to expand and accelerate the discoveries that will one day allow VA to offer personalized health care to a Veteran's specific genetic traits and military experiences.
- ORD supports Actively Managed Portfolios (AMPs) to align research awards with Congressional and Presidential priorities. These include Suicidal Prevention, Traumatic Brain Injury, Military Exposures, Precision Oncology, Pain and Opioid Use, and Gulf War Illness. Using a strategy to integrate research with clinical care across the enterprise, these AMPs aim to create communities of research and care to better shape how VA can take evidence-based approaches focused on the issues most important to Veterans.
- VA's Trustworthy AI Framework established a robust set of principles to ethically guide the entirety of the AI lifecycle from development to implementation. Early success in VA's AI research have enhanced the detection of precancerous polyps, identified Veterans at a high risk of

suicide, and better managed opioid use among Veterans. AI tools currently in development will further empower clinicians and enhance outcomes for Veterans in a wide variety of fields.

4. Notable Researchers and Projects:

- **Dr. Rory Cooper** is the director and founder of the Human Engineering Research Laboratory (HERL), a VA and University of Pittsburgh collaboration, which researches wheelchair designs, biomechanics and more. For his innovations in mobility technology, he has been inducted into the National Inventors Hall of Fame and awarded the National Medal of Technology and Innovation. HERL holds patents on 25 innovations in wheelchair designs, with new innovations being developed every year.
- **Dr. Ziyad Al-Aly** is the chief of research and development at VA's St. Louis Healthcare System and one of the foremost experts on COVID-19 in the world. His research made several groundbreaking discoveries on the virus, the effects of the vaccine, and the long-term after-effects. In 2023, Altmetric ranked Dr. Al-Aly's work in the top ten of 23 million publications, having been cited over 100,000 times, and in 2024 he made Time Magazine's Top 100 list of people who have most influenced global health. He continues to serve as an epidemiological expert in Long Covid research but has recently expanded his work to study the risks and benefits of GLP-1 weight loss drugs.
- **Dr. Matthew Rettig**, chief of the Division of Hematology-Oncology at VA Greater Los Angeles, has personally brought in \$50 million from external sources to boost VA's precision oncology program through his hard work and reputation as a preeminent researcher in prostate cancer, the second most lethal form of cancer among Veterans. He's discovered multiple biomarkers for cancer, including some from toxic military exposures, and developed a database of over 1.2 million entries with 300 searchable fields to accelerate cancer research across the field. He is currently developing an AI algorithm to predict the possible recurrence of high-risk prostate cancer, a condition that represents one-fifth of all new cases.
- **Dr. Robert Kirsch**, executive director of the Center for Functional Electrical Stimulation at the Cleveland VA, is an internationally recognized leader in rehabilitation and neural engineering research. By combining functional electrical stimulation with brain-computer interfaces he and his research team were able to implant electrodes in the brain and upper arm to re-activate paralyzed muscles and restore upper limb movement to Veterans with spinal cord injuries. VA continues to refine this work across multiple centers, recently brain mapping dozens of complex hand gestures.



TAB 3

208 (Conflict of Interest)

HVAC Subcommittee on Health – Talking Points

SECTION 208

BLUF:

- SVAC Majority has reintroduced draft legislation originally proposed in the 118th Congress to address the 208 issue.
 - OCLA is reviewing the legislation.
 - OCLA legislative team is speaking with OMB about a path forward outside of legislation. We are awaiting updates.
- The OGC Designated Ethics Official has approved a 9-month extension of the 208-implementation date with a review at 6 months.
- VA continues to develop a technology-assisted waiver evaluation tool.
 - The tool will be highly complex with over 160 possible permutations.
 - Beta test is expected to take several months.
- System Development Timeline (from Gery):
 - Primary system testing: November 2025 to January 2026
 - End User Validation: January – February 2026
 - System Refinement: February – March 2026
 - Field and OGC Training: April 2026
 - **Official Launch: June 2026**

Statistics:

- ORD anticipates upwards of 4,000 waivers required each year.
 - Every researcher submitting an application to NIH or other funder will have to have a waiver.
 - Review for potential waivers are required for each project submitted regardless of funding source.
 - Waivers are specific to a project, so a researcher submitting multiple applications to NIH in a given year will have to have a waiver for each application.
 - Waivers will be spread out over the fiscal year aligned with funding cycles.
- By contrast, under the previous rule, VA issued only nine § 208 waivers in 2023.

Budget:

None.

Additional Talking Points:

- Background: VA's Office of General Counsel (OGC)'s revised interpretation of "particular matter" under 18 U.S.C. § 208 presents a significant conflict of interest

SECTION 208

(COI) challenge for VA researchers and educators holding dual appointments. This interpretation could necessitate thousands of waiver requests, impose considerable administrative burdens, and, in the worst case, lead to the departure of VA researchers and educators and jeopardize our relationships with Academic Affiliates (Affiliates) and VA Nonprofit Research and Education Corporations (NPCs).

- Anticipated Impact: The new interpretation reverses 62 years of established practice and interpretation. It will impede dually appointed researchers from starting or continuing their work and will lead to burdensome administrative delays, as the statute does not account for the unique circumstances these researchers face.
 - Under the previous rule, the scientific and funding components of a research grant application were assessed separately, allowing researchers to engage in the scientific aspects while being excluded from the business or fiscal management components. However, under the new rule, the scientific and funding aspects of the grant will be regarded as inseparable. This means that dually appointed researchers would be involved in a “particular matter” that could affect their employer’s financial interests, thus violating § 208.
 - Nearly 90% of the 6,600 VA researchers hold dual appointments. The VA Office of General Counsel has determined that these researchers will need to obtain a § 208 waiver before they can even submit a research project for funding consideration. As a result, VA anticipates needing approximately 4,000 waivers each year once the new rule takes effect. This volume will be operationally unsustainable and could jeopardize the administration of the VA Research Program.



TAB 4

Alternative Therapies – Mental Health

HVAC Subcommittee on Health – Talking Points

ALTERNATIVE THERAPIES – MENTAL HEALTH

BLUF:

- Psychedelics and HBOT are addressed separately (see tabs)
- Beyond Psychedelics and HBOT, Ketamine and Esketamine treatments were incorporated into recommendations in the 2022 VA/DoD CPG for Major Depression and are considered standard of care.
 - VA has gone from 8 facilities offering these treatments 10 years ago to now offering them in 49 facilities across the country.
- Another alternative therapy is Stellate Ganglion Block (SGB).
 - VA has gone from 8 facilities offering these treatments 10 years ago to now offering them in 49 facilities across the country.

Statistics:

Budget:

Additional Talking Points:

1. VA Research Contributions in Mental Health (MJS)

Longer-Term Contributions:

- **VA investigators have led the development of numerous clinical practice guidelines** (e.g., 2023 VA/DoD Clinical Practice Guideline for PTSD). VA research contributed prominently to recommendations.
- **VA research has set the global standard for how PTSD is screened and diagnosed.**
 - Developed the Primary Care PTSD Screen (PC-PTSD); used nationally to screen for PTSD in the VA.
 - Developed the PTSD Checklist (PCL); used to assess PTSD symptom severity in VA's measurement-based care initiative and in mental health programs.
 - Developed the Clinician-Administered PTSD Scale (CAPS); the gold standard for PTSD diagnosis used around the world in both clinical care and research.
- **VA investigators were the first to establish an effective treatment for female veterans with PTSD.**
 - Established prolonged exposure therapy as an effective treatment for PTSD in female veterans and active-duty military personnel.

HVAC Subcommittee on Health – Talking Points

ALTERNATIVE THERAPIES – MENTAL HEALTH

- Citation: Schnurr PP, Friedman MJ, Engel CC, et al. Cognitive Behavioral Therapy for Posttraumatic Stress Disorder in Women: A Randomized Controlled Trial. *JAMA*. 2007;297(8):820–830. doi:10.1001/jama.297.8.820
- **VA research identified clinical practices that may be unnecessary**
 - The addition of lithium to usual mental health care did not reduce the incidence of suicide-related events in veterans with major depression or bipolar disorders who experienced a recent suicide event.
 - Citation: Katz IR, Rogers MP, Lew R, et al. Lithium Treatment in the Prevention of Repeat Suicide-Related Outcomes in Veterans with Major Depression or Bipolar Disorder: A Randomized Clinical Trial. *JAMA Psychiatry*. 2022;79(1):24-32. doi:10.1001/jamapsychiatry.2021.3170

More Recent Contributions:

- **Completed the largest PTSD psychotherapy trial ever conducted (CSP #591).**
 - Rare head-to-head trial comparing the effectiveness of prolonged exposure (PE) vs cognitive processing therapy (CPT).
 - PE was statistically more effective than CPT, but the difference was not clinically significant, and improvements in PTSD were meaningful in both treatment groups.
 - The results suggest that Veterans have options for their treatment preferences. Authors recommend shared decision-making to help Veterans understand the evidence and select their preferred treatment.
 - Citation: Schnurr PP, Chard KM, Ruzek JI, et al. Comparison of Prolonged Exposure vs Cognitive Processing Therapy for Treatment of Posttraumatic Stress Disorder Among US Veterans: A Randomized Clinical Trial. *JAMA Netw Open*. 2022;5(1):e2136921. Published 2022 Jan 4. doi:10.1001/jamanetworkopen.2021.36921

HVAC Subcommittee on Health – Talking Points

ALTERNATIVE THERAPIES – MENTAL HEALTH

- **Groundbreaking Study Mapped PTSD at a Cellular Level in Veterans' Brains**
 - Critical step towards unraveling the biological underpinnings of PTSD
 - Shows how the disorder alters signaling pathways, inflammatory responses, and even basic cellular function.
 - Brings the field one step closer to precision medicine approaches that are grounded in the biology of the disorder
 - VA research Currents article - <https://www.research.va.gov/currents/0725-VA-Enters-into-Next-Phase-of-PTSD-Research.cfm>
 - Citation: Hwang, A., Skarica, M., Xu, S. *et al.* Single-cell transcriptomic and chromatin dynamics of the human brain in PTSD. *Nature* **643**, 744–754 (2025). <https://doi.org/10.1038/s41586-025-09083-y>
- **Ongoing strategic initiative to accelerate the development of better PTSD medications for Veterans.**
 - Fourteen clinical trials funded under PTSD Psychopharmacology Initiative with the goal of impacting Clinical Practice Guidelines and US Food and Drug Administration labeling.
 - Most of these trials will be completed in the next 2 years
 - Potential to improve medication treatment options for veterans.
- **VA is studying psychedelic-assisted therapy for PTSD and other mental health conditions**
 - To date, three psychedelic trials have been selected for funding by ORD.
 - Wolfgang AS, McClair VL, Schnurr PP, et al. Research and implementation of psychedelic-assisted therapy in the Veterans Health Administration. *Am J Psychiatry*. 2025;182(1):17-20. doi:10.1176/appi.ajp.20240751

2. Brain Health (MJS)

The ALS (Amyotrophic Lateral Sclerosis, Lou Gehrig's disease) Biorepository Brain Bank: this ORD-funded brain bank is one of the largest ALS brain banks in the world. Over 400 Veterans have donated their brains for research to find the cure and ease suffering of their fellow Veterans.



TAB 5

Artificial Intelligence (AI)

HVAC Subcommittee on Health – Talking Points

ARTIFICIAL INTELLIGENCE

BLUF:

- Current and Future AI Research: The VA AI Strategy prioritizes R&D as a core pillar.
- AI research spans a spectrum of foundational innovation in building and adapting algorithms and technologies for healthcare to exploring effective ways to integrate AI into clinical, research, and administrative practice.
- VA research leverages academic affiliates to recruit and support AI expertise.
- VA healthcare remains on cutting edge through partnerships with VAMC and technology researchers.
- Controls for Data Safety: VA ensures AI research safeguards Veteran data, privacy, and civil rights guided by relevant Federal laws.
- ORD is considering other transactional authorities (OTA) to accelerate critical AI capabilities and investments in timely and strategic ways.
- ORD is building technical infrastructure supporting ORD researchers to accelerate development, adoption, use, and evaluation of AI capabilities.
 - This encompasses the data ecosystem as well as data interoperability enabling data exchanges across VA systems, the private healthcare sector, and external partners (e.g., DoD, NIH, etc.).
 - It also includes computing resources (e.g., High Performance Computing via Department of Energy, and Cloud-service providers).
 - Model development and deployment platform capabilities will continue to be evaluated with our OIT partners so promising AI capabilities can be efficiently translated into clinical practice.

Statistics:

- ORD has over 390 AI-related projects representing ~\$113M funded from the portfolio. This represents ~315 unique VA investigators spanning ~59 VA sites. The total funding represents total awards for research efforts – a subset of the funding supports AI-enabling capabilities and efforts. We anticipate this trend to continue to increase.
 - It is important to acknowledge that this number includes AI being used for statistical inference (association studies, etc.) as well as AI that is diagnostic, predictive, or generative (risk models, large language models, etc.).
- AI-related research projects focus in areas that directly target the most important health problems facing Veterans and the VA healthcare providers.
- Potential benefits to Veterans: AI has the potential to transform and scale healthcare for over 9 million Veterans.

HVAC Subcommittee on Health – Talking Points

ARTIFICIAL INTELLIGENCE

Budget:

Additional Talking Points:

- ORD has supported and continues to partner to increase the research community's access to computing, models, and software resources as part of the NAIRR pilot and is supported by the AI Action Plan.
- ORD is actively identifying opportunities to align to support scaling NAIRR as well as the VA ORD research community through connecting critical AI resources, lessons learned, and best practices to ensure the most promising insights are identified, validated, and brought back to the Veteran community.
- ORD has actively contributed and supported the upcoming publishing of the new National AI Research and Development Strategic Plan, led by OSTP, in an effort to foster the next generation of AI breakthroughs. We recognize that this will help guide federal AI research investments. We anticipate this to be published in December.
- Informed Consent & Regulatory Reviews: Before AI research involving Veterans as study participants may be conducted, the research undergoes heightened regulatory review scrutiny to ensure the confidentiality of Veteran data and the appropriate use of AI software (including AI associated with medical devices) to protect Veteran's health and safety. As well, for direct patient interventions consent and authorization of Veterans who are study participants must be obtained and documented.
- Data Safety & Privacy Considerations: Protecting Veteran data is paramount – security, ethics, and privacy compliance is paramount. ORD is responsible for ensuring key laws, policies, and directives guiding protecting Veteran health data include are followed in the conduct of research, including AI. (see below for details)

Reference Materials

- HIPAA: limits disclosure, mandates safeguards, and grants access rights through establishing standards for use and disclosure of PHI, security safeguards (across technical, physical and ad), and patient rights to access, correct, and restrict their data. 45 CFR160 and 164 regulations provide guardrails for the privacy, security, and notification rules.
- 38USC7332: requires consent for disclosure for sensitive health data (HIV, SUD), and limiting internal sharing of data.
- 38USC5705: quality assurance data which protects peer review/QA records from public disclosure.

HVAC Subcommittee on Health – Talking Points

ARTIFICIAL INTELLIGENCE

- VA Handbook 6500 series: security of electronic records through cybersecurity and access control standards.
- 38CFR0.605 (VA Ethics Code): guides ethical data use requiring Veteran-centered, fair, transparent data practices.
- HITECH/HIPAA Omnibus/6502: guides breach response and accountability and requires notification, penalties, and mitigation of breaches.
- Evidence Act, Public Law 115-435: aims to improve how VA (including other federal agencies) use data, evidence, and evaluation to inform policy-making by building robust data governance structures, promote open and secure access to data, creating learning agendas and evaluation plans, designate a CDO, and support evidence-building activities, including statistical and evaluation research.
- Federal AI EOs Executive Actions:
 - EO14296 (MAY 9, 2025): Keeping Promises to Veterans and Establishing a National Center for Warrior Independence
 - EO14179 (JAN 23, 2025): Removing Barriers to American Leadership in AI
 - EO14303: (MAY 23, 2025): Restoring Gold Standard Science
 - EO14292: (MAY 5, 2025): Improving the Safety and Security of Biological Research
 - EO14243: (MAR 20, 2025): Stopping Waste, Fraud, and Abuse by Eliminating Information Silos
 - AI Action Plan (July 2025)
 - OMB M25-21: VA is in compliance with the OMB to ensure any efforts leveraging AI will be guided by responsible considerations to include access/documentation for authoritative data sources needed for training, testing, and validation. The solutions include enterprise data platforms and catalogs (e.g., CDW, VA Enterprise Cloud, VINCI, etc.). This is critical as training and validation of models is core to R&D AI related projects.



TAB 6

Clinical Trials

HVAC Subcommittee on Health – Talking Points

CLINICAL TRIALS

BLUF:

- Access to high quality clinical trials is one of ORD's THREE strategic priorities.
- As every world-class cancer care system does, we are committed to offering our Veterans the option to participate in trials.
 - When a Veteran has reached the limit of what standard medical practice can offer her, clinical trials offer them hope.
 - We work hard to make joining a clinical trial easy for Veterans.
 - We cannot ensure that Veterans in remote areas join clinical trials. We can only make it easier to try to recruit them and make it more appealing to join.
- Partnership has been a central part of our strategy to increase Veterans' access to clinical trials.
 - We are actively collaborating with the VA non-profit corporations and NAVREF, their professional organization; the National Cancer Institute; the National Institute on Aging; the National Institute for Allergy and Infectious Diseases; among many others.
- Goal is that all Veterans, no matter where they live, get access to trials.
 - When the Veteran cannot come to the clinical trial, we want to be able to bring the clinical trial to the Veteran whenever it is safe to do so.

Statistics:

- 2,000+ active VA clinical trials (includes both ORD and non-ORD funded trials)
- 462 active (FY25) ORD-funded trials
 - 96 Brain, Behavioral, and Mental Health Broad Portfolio
 - 66 Health Systems Research Broad Portfolio
 - 43 Medical Health Broad Portfolio
 - 148 Rehabilitation Research, Development and Translation Broad Portfolio
 - 7 Gulf War Research
 - 1 Military Exposure Research Program
 - 21 Pain and Opioid Use Actively Managed Portfolio
 - 15 Precision Oncology Actively Managed Portfolio
 - 18 Suicide Prevention Actively Managed Portfolio
 - 13 Traumatic Brain Injury Actively Managed Portfolio
 - 34 Cooperative Studies Program
- 10,000+ anticipated Veterans to enroll in ORD-funded clinical trials currently open to enrollment

HVAC Subcommittee on Health – Talking Points

CLINICAL TRIALS

Budget:

- \$143.8 million in FY25 for ORD clinical trials
 - \$18.5m Brain, Behavioral, and Mental Health Broad Portfolio
 - \$15.7m Health Systems Research Broad Portfolio
 - \$10.7m Medical Health Broad Portfolio
 - \$26.5m Rehabilitation Research, Development and Translation Broad Portfolio
 - \$246k Gulf War Research
 - \$173k Military Exposure Research Program
 - \$6m Pain and Opioid Use Actively Managed Portfolio
 - \$9m Precision Oncology Actively Managed Portfolio
 - \$3.3m Suicide Prevention Actively Managed Portfolio
 - \$1.3m Traumatic Brain Injury Actively Managed Portfolio
 - \$52.3m Cooperative Studies Program

Additional Talking Points:

Highlighted CSP Trials:

1. CSP 2017: The VA Osseointegration Study – Priority Area = **Novel Prosthetics**
 - CSP 2017 is a prospective, open label, randomized multi-center study using a novel VA designed Percutaneous Osseointegrated Prosthesis (POP) device for direct skeletal prosthetic limb attachment in persons with transfemoral level lower limb amputations.
 - This trial is on-going surgeries occurring at 2 (soon to be 6) VA medical centers.
 - Impact: Improved movement and quality of life for veterans using socket suspense devices.
 - This trial represents the first time that CSP and ORD were able to negotiate long-term return on investment for one of its study collaborations with a device manufacturer. The VA will receive substantial price reductions on all future devices purchased until the initial cost of the study is recouped, and will receive modest savings afterward.
2. CSP 2018: The GRIT Study – Priority Area = **Traumatic Brain Injury**
 - CSP 2018 is a placebo-controlled, double-blind, parallel-group randomized clinical trial (RCT) of recombinant human growth hormone (rhGH) replacement therapy vs. placebo in Veterans with deployment-related mild traumatic brain injury (mTBI).

HVAC Subcommittee on Health – Talking Points

CLINICAL TRIALS

- This study will assess the efficacy of rhGH as well as treatment safety and compliance.
 - Impact: The impact of rhGH on a Veteran population in the setting of mTBI has never been studied. The study will set the standard for treatment of mTBI.
 - If rhGH is found to be safe, and effective in improving the outcomes measured, this study is likely to transform clinical practice by delivering a new treatment option to a patient population that may number in the tens of thousands within the VA, and hundreds of thousands worldwide.
 - If the effect is not shown, the study's impact will benefit VA clinical care by providing the VA precious opportunity to consider the appropriateness of this time-consuming diagnostic work-up and treatment.
3. CSP #2005: The VALOR Study – Priority Area = **Cancer Treatment** (Access to treatment as well)
- CSP 2005: The **Veterans Affairs Lung cancer surgery Or stereotactic Radiotherapy (VALOR)** Trial is a prospective randomized trial to evaluate the role of stereotactic body radiation therapy (SBRT) for patients (participants) with early-stage lung cancer who are usually treated with surgery.
 - Largest trial of lung cancer treatment ever done.
 - Impact: Lung CT screening is expected to more than double the number of patients who will present with stage I lung cancer. This trial will help clinicians understand whether SBRT provides an acceptable alternative to surgery for this population and whether it has the potential to increase access to care and help to save lives.
4. CSP #2041: The CRAVE Trial – Priority Area = **Alcohol Use Disorder**
- Randomized, double-blind, placebo-controlled phase 3 clinical trial is to evaluate the efficacy of semaglutide, a GLP-1 receptor agonist (RA), compared to placebo in reducing heavy drinking in the treatment of moderate to severe alcohol use disorder (AUD).
 - The primary outcome is no heavy drinking days (NHDD) in the last 4 weeks of the 20-week intervention period (weeks 17 to 20).
 - The study seeks to enroll 376 Veterans at 15 VA medical centers. The planned study duration is 48 months.
 - Kickoff anticipated for calendar year 2026.



TAB 7

Data and Safeguards

HVAC Subcommittee on Health – Talking Points

DATA AND SAFEGUARDS

BLUF:

- ORD is committed to sharing its data to the extent possible to advancing scientific research at improving Veteran health care.
 - We participate in the Gold Standard Science and Public Access to Research led by OSTP.
 - We established an IAA with PubMed to include VA funded publications - 29,500 pubs to date.
 - ORD-funded clinical trials must be registered in clinicaltrials.gov.
- ORD protects Veterans data (and the data of other agencies) and ensures only appropriate access and use of these data.
- ORD works with Veterans to help us develop and further refine our data use and sharing policies and guidelines.
- ORD offers computational platforms and tools for VA researchers and increased data storage and processing capacity.
 - The VA Informatics and Computing Infrastructure (VINCI) is a secure, centralized platform that provides researchers with remote and secure access to essential VA data, applications, and services.
 - ORD established a privacy-preserving records linkage (PPRL) hub in VINCI to deidentify and link sensitive VA data with external data while ensuring individuals' privacy.
- ORD has data sharing partnerships with CMS and DoD, which include medical claims and health records of servicemembers and veterans.
 - These partnerships provide a more comprehensive understanding of care for veterans while ensuring privacy and data security.

Statistics:

- ORD supports the use of data on 23.5 million veterans. Of these, 16.6 million have received care from the VHA at some point, and 7.0 million have received care in the past 12 months.

Budget:

Additional Talking Points:

- VA ORD has piloted secure multiparty computation (MPC) to allow multiple organizations to collaborate on shared analyses without exchanging raw data,

HVAC Subcommittee on Health – Talking Points

DATA AND SAFEGUARDS

enhancing privacy and compliance with HIPAA, ultimately improving research and public health insights.

- Collection, analysis, storage and sharing of Veterans' data for research purposes is governed by many federal statutes and regulations and VA policies, including restrictions on data use and disclosure, requirements for prior review and approval, consent or authorization by individuals or approved waivers, data use agreements between VA and non-VA institutions, and required reporting and mitigation of breaches and unauthorized disclosures.
- Unless expressly excepted by law, researchers must obtain a Veteran's consent or authorization before the Veteran's data may be used or disclosed for research purposes, and the method for obtaining and documenting a Veteran's consent or authorization must first be reviewed and approved by a specially designated privacy officer, board or research review committee to ensure that the Veteran is adequately informed about how their data may be used in research.
- In circumstances when Veterans' consent or authorization may be waived under law, for example when the Veteran is lost to contact, use or disclosure of Veterans' data for research purposes is subject to greater scrutiny and limits, such as the use or disclosure presenting only minimal risk to the data's confidentiality and Veteran's privacy, that the researcher could not practicably be conducted without the data, and that the data are deidentified. Privacy officer, board or research review committee review is still required to ensure that the more stringent requirements are satisfied.



TAB 8

Harvard Research

HVAC Subcommittee on Health – Talking Points

HARVARD RESEARCH

BLUF:

- ORD supports intramural research within the VA and does not have the authority to fund grants to external organizations. As such, VA does not fund VA research grants at Harvard University.
- However, VA-funded researchers may obtain specialized expertise in bioinformatics and methodologies for medical science through contracts with Harvard University.
- At the request of the administration/agency leadership, ORD conducted a review of existing contracts with Harvard University through a data call to the field and working with the Contracting Offices and Contracting Officer's representatives and provided reports to the VA Senior Advisors.
- At this time, VA has approved nearly all contracts with Harvard University that support VA-funded research projects, including projects on prostate cancer screening, military exposures, and suicide prevention, to move forward (Stats below)

Statistics:

- There was a total of 7 contracts that VA had with Harvard University:
 - Five were in support of projects at the Boston VA Health Care System, including the Million Veteran Program, advanced methodologies for generating evidence for clinical decisions for a number of health conditions (VA-CAUSAL), military exposures research and suicide prevention.
 - One subaward at the Bedford VA health Crae System in support of research studying the association of military exposures and ALS.
 - One contract at the Ann Arbor VA medical center comparing quality of cancer care at the VA with that in community care
 - Only the contract at Ann Arbor VA was terminated. Two contracts (VA CAUSAL and suicide prevention were approved to complete the current option years on the contract), and remaining work will continue within the VA. The remaining 4 contracts were approved to move forward for the entire contract period.

Budget:

Additional Talking Points:



TAB 9

HBOT (Hyperbaric Therapy)

HVAC Subcommittee on Health – Talking Points

HBOT

BLUF:

- HBOT is recognized for conditions like decompression sickness and wound healing, however, the FDA has not authorized its use for PTSD or TBI treatment.
- VA and DoD Clinical Practice Guidelines (CPGs) found insufficient evidence to recommend HBOT for PTSD and strongly recommend against its use for mild TBI.
- Published results of scientifically rigorous VA and DoD research on TBI have repeatedly shown that HBOT has the same impact as a placebo and no clinically relevant long-term effects.
 - In addition to the lack of patient improvement, the use of HBOT after a severe TBI may have harmful impacts, including seizures.
 - [Due to 100% oxygen inside the hyperbaric chamber, there are risks of accidents.](#) (link goes to article)
 - In 2025 alone, at least two deaths caused by fire inside the chamber have been reported. A 5-year-old Michigan boy being treated off-label for attention deficit-hyperactive disorder and a 43-year-old healthcare provider in Arizona receiving treatment for an unknown condition.
- VA continues to closely monitor the research on HBOT, and none of the recently published studies have changed our recommendations at this time.

Statistics: None to report

Budget: No planned funding

Additional Talking Points:

[List of conditions for which hyperbaric chambers are cleared for marketing by the FDA.](#)



TAB 10

Mental Health

HVAC Subcommittee on Health – Talking Points

BRAIN AND MENTAL HEALTH – SCOTT HANNON INITIATIVE FOR PRECISION BRAIN AND MENTAL HEALTH

BLUE:

- Under the Hannon Act, Section 305, the ORD's Scott Hannon Initiative for Precision Brain and Mental Health (SHIPBMH) initiative will identify and validate biomarkers for depression, psychosis, anxiety, PTSD, bipolar disorder, and TBI.
- SHIPBMH will utilize machine learning algorithms to integrate multiple biomarkers (neuroimaging, biofluids, EEG, and genetic and neurobehavioral assessments) to provide a clinically actionable diagnostic solution.
- This effort will leverage discoveries from ongoing research programs such as MVP, Long-Term, Impact of Military-Relevant Brain Injury Consortium - Chronic Effects of Neurotrauma Consortium, the Translational Research Center for TBI and Stress Disorders (TRACTS) and the new research area on the brain health effects of military occupational blast exposure.

Statistics: LIMBIC-CENC reports:

- Over a third of Veteran participants with TBI histories have PTSD
- 65% of those participants have mild to severe depression.
- Veterans with diagnosed TBI and or had a military occupation with known risks for occupational related blast exposures have elevated risk for a range mental health conditions.

Budget: \$10.8M for associated VA projects for FY2026. Additional non-VA awards are to be funded.

Additional Talking Points:

Precision Mental Health - Hannon Act Section 305:

- In response to Section 305 requirements, we have launched a precision brain/mental health initiative.
 - centered around the concept of "total brain diagnostics"
 - goal to create clinically actionable diagnostic, predictive and treatment responsive machine learning algorithms.
- Selected 4 proof of concept studies using multimodal biomarkers for funding
 - biomarkers include EEG and imaging

HVAC Subcommittee on Health – Talking Points

BRAIN AND MENTAL HEALTH – SCOTT HANNON INITIATIVE FOR PRECISION BRAIN AND MENTAL HEALTH

- has successfully harmonized multimodal biomarker data between two large longitudinal cohorts of Veterans with TBI and/or mental health conditions, as well as those w/ neither TBI nor MH condition
- Established Brain Health Coordinating Center (BHCC) in February of 2025
 - The BHCC will act as a central hub to enhance the VHA's Learning Health System by creating an integrated Brain Health Network.
 - The BHCC will focus on four key pillars to drive this mission:
 1. Engagement – Strengthening communication and collaboration across the VA community and with external partners;
 2. Quality Improvement – Advancing best clinical practices through continuous, data-informed improvement;
 3. Research – Supporting innovative investigations that drive better outcomes;
 4. Education – Enhancing clinical knowledge and practice through ongoing learning.



TAB 11

Military Exposure

HVAC Subcommittee on Health – Talking Points

MILITARY EXPOSURE

BLUF:

- VA/ORD seek to understand the health impact of military service environmental and occupational exposures (MSEE) and use that to inform care and policy.
- ORD has a duty to do our part in coordination with HOME.
 - Our Gulf War research advisory committee has a strong Veteran engagement component.
 - MERP has a strong engagement with HOME to support DoD studies.
- The MERP provides the capabilities to better answer questions about MSEE.
 - We established research infrastructure to better support MSEE research. Rather than treat the symptoms, we are committed to treating the system.
- PACT Act (Aug 2022) increased focus on Military Exposure (ME) research:
 - Addressing toxic exposures for our Veterans is a mission that requires interagency collaboration, just as battle against COVID did. The Toxic Exposures Research Working Group (TERWG) is Mapping the path forward, And make no mistake, that map needs to be well-funded to lead us to real solutions.
- Three main areas of MSEE:
 - Informing drug development: Basic science identifies new mechanisms of toxic dysfunction. This information is used to identify existing commercial drugs for interventions or publish findings to inform commercial pharmaceutical drug development efforts.
 - Finding Veterans at Risk: Epidemiological studies inform VA how disease is spread and who is most vulnerable. Epidemiological research identifies Veterans who are at risk for specific adverse health outcomes.
 - Clinical treatments: Research of clinical treatments is the most visible extension of research, backed by basic science and epidemiological research.

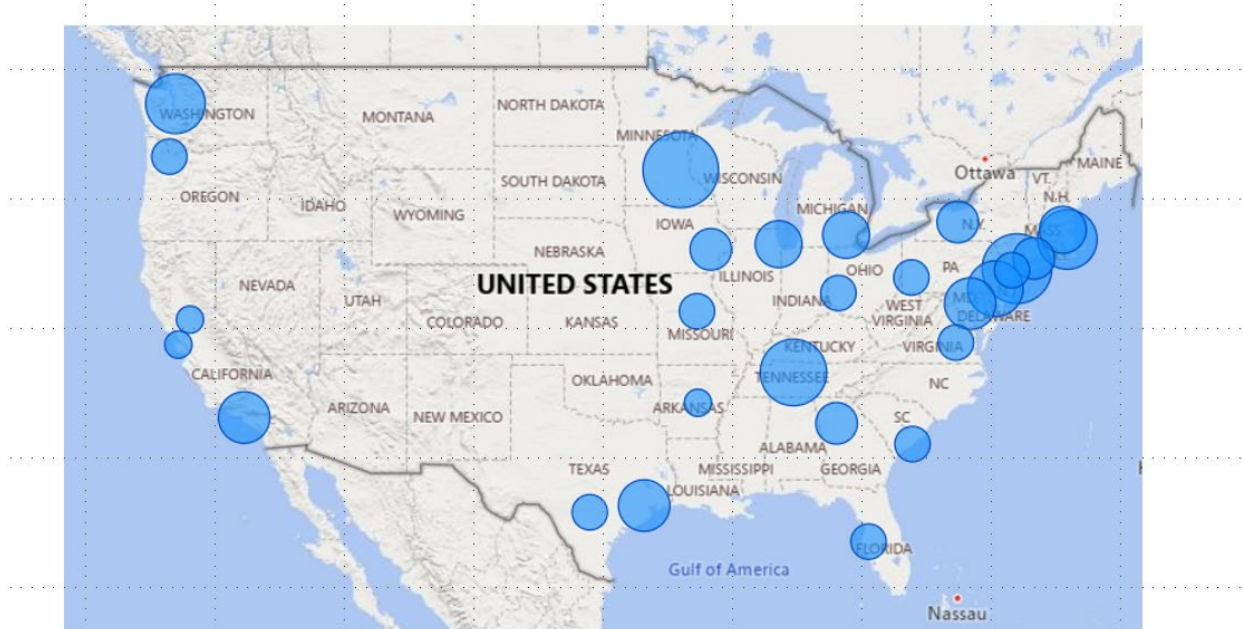
HVAC Subcommittee on Health – Talking Points

MILITARY EXPOSURE

Statistics:

MERP AMP Quick Look of TEF Funded Projects				
Projects in JIT	Projects Ending in FY25		Projects Starting in FY25	
15	11	\$5,066,479	26	\$14,558,802
	Count of Index	Sum of Budgeted	Count of Index	Sum of Budgeted
Active Clinical Trials	Projects Ending in FY26		Projects Starting in FY26	
1	5	\$4,421,900	(Blank)	(Blank)
	Count of Index	Sum of Budgeted	Count of Index	Sum of Budgeted

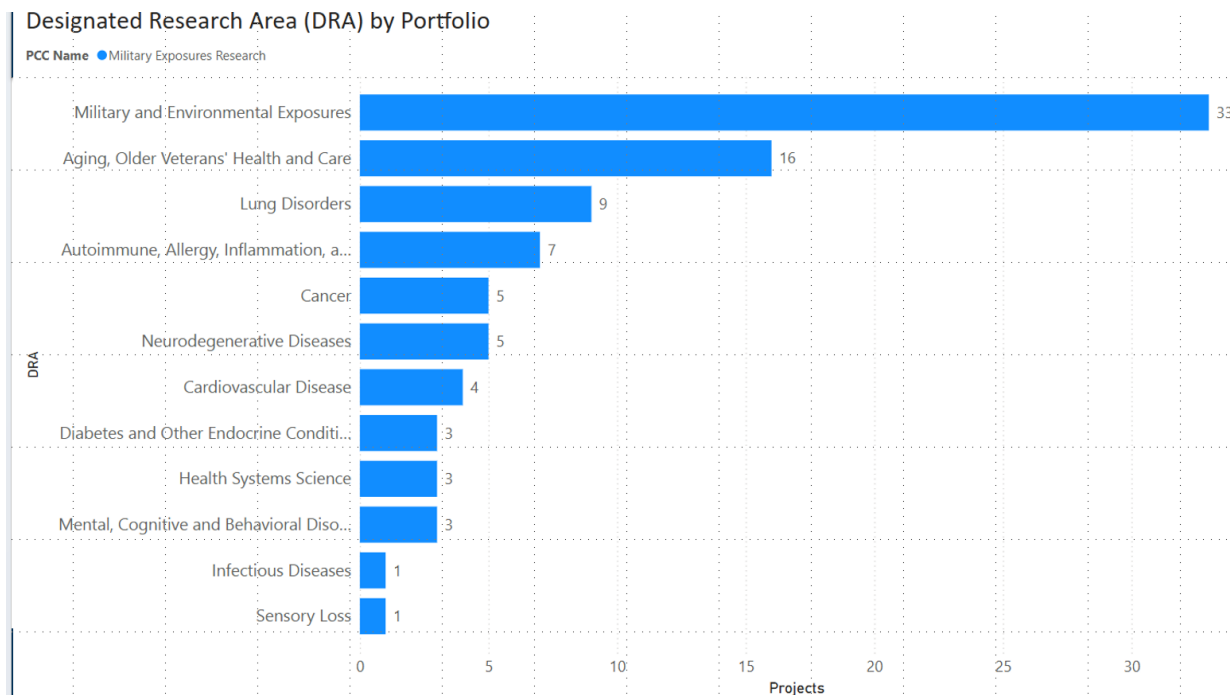
Map of MERP funded projects. Circles are for each project, and size of the circle is proportional to dollars obligated per project.



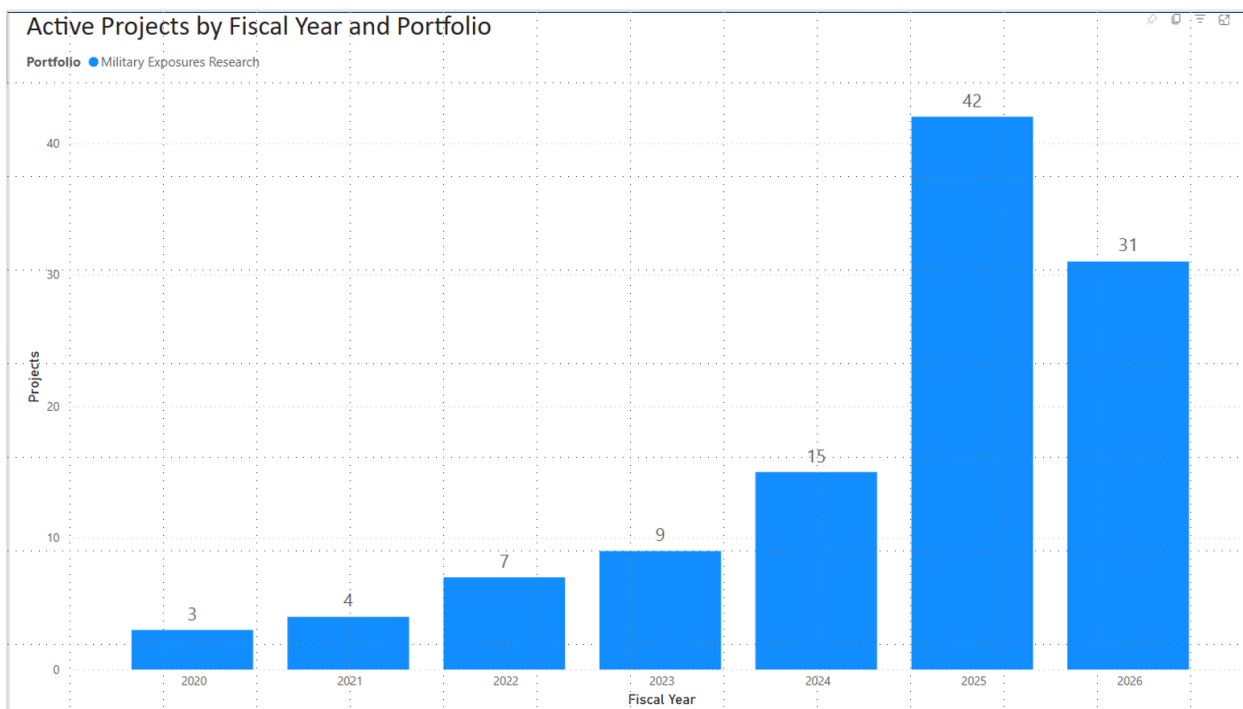
HVAC Subcommittee on Health – Talking Points

MILITARY EXPOSURE

These are MERP research activities by key words (DRA) from RAFT.



Growth of projects from MERP. 2026 reflects projects expected to complete JIT. 2026 does not include new applications.



HVAC Subcommittee on Health – Talking Points

MILITARY EXPOSURE

Budget: Toxic Exposure Funding (mandatory appropriation):

FY23: \$2.48M

FY24: \$46M

FY25: \$59M

FY26:

Increase in projects is tracking increase in funding.

MERP/GWI are looking for consolidated projects and centers to accelerate research. These are being posted through the new ISRM I50 mechanism to accomplish this administratively.

Additional Talking Points:

MERP is:

- (1) Curating data and specimens of the Vietnam Veteran Air Force Health Study to evaluate generational, cancer, and other health concerns of this cohort, in consultation with Vietnam Veterans of America and other stakeholders.
- (2) Expanding training resources offered by the MERP. These resources will include webinars, online resources, and optimized customer service activities to support VA field investigations (e.g., laboratory, epidemiology, and medical studies).
- (3) Completing first year of operations of the Vietnam Veteran Air Force Health Study (also known as the Ranch Hand Study) in VA Science and Health Initiative to Combat Infectious and Emerging Life-Threatening Diseases (VA SHIELD). This study will include archiving and distributing biospecimens and associated data.

Military Service Environmental Exposures (MSEE)

Military toxic exposures stem from environmental sources, occupational specialties, and related health outcomes observed in Veterans. Forty-five percent of Veterans who underwent a toxic exposure screen reported experiencing at least one toxic exposure during their military deployment and expressed concerns about potential health impacts for themselves and/or their children. The toxic exposures of greatest concern include burn pit fumes and ash, Agent Orange, firefighting foams, and industrial chemicals that accumulate in human tissue. Adverse health outcomes include disorders affecting major organ systems, particularly the cardiovascular, neurological, and digestive systems, as well as the skin and eyes. MSEE studies aim to (1) inform drug treatments, (2) identify Veterans at risk, and (3) develop bedside clinical treatments.

HVAC Subcommittee on Health – Talking Points

MILITARY EXPOSURE

Ensuring appropriate representation of Veteran populations and maintaining unbiased peer-review are crucial for the highest quality MSEE research. Studies focusing on specific deployments, women's health, and particular Military Occupational Specialties (MOS) may necessitate the involvement of multiple VA medical centers. The Office of Research and Development (ORD) collaborates with cross-agency specialists from the Health Outcomes Military Exposures (HOME) and the War Related Illness and Injury Study Centers (WRIISC).



TAB 12

MVP

HVAC Subcommittee on Health – Talking Points

MILLION VETERAN PROGRAM

BLUF:

- One of the world's largest research programs on genes, lifestyle, military experience, exposures and health; over 1,090,780 Veterans enrolled.
- Every Veteran across race, sex, age, ability, and service branch invited to join (See Stats)
- MVP facilitates high quality research in areas that matter to Veterans using genetic, and health data; goal of moving scientific discoveries into the clinic for precision healthcare to Veterans (See Stats for types of data and examples of research and clinical translation under Additional talking Points)

Statistics:

- Genomic and health record data from more than 1,080,000 Veterans
 - 90% male, 10% female
 - 25% Veteran participants representing racial and ethnic minorities
 - 8% Hispanic (~80,000)
 - 18% Black (~180,000, **largest research database in the world of African ancestry**)
- Current MVP projects:
 - 120+ projects and over 475 publications to date
- Data generated from Veterans blood samples include:
 - **Over 1,000,000 samples have been genotyped**
 - Genotype data from 650,000+ samples have been validated and are available for research.
 - **140,000+ whole genome sequences completed**
 - 102,000 whole genomes are available to VA researchers, and the remaining are undergoing quality control prior to release.
 - **Metabolomic analyses on 80,000 samples** are underway and will be made available to researchers in the future.
 - **Methylation data on 40,000 samples** are available to researchers in 2024.

Budget: ~\$90.3M in FY25 (includes the \$13.25M additional funding for sequencing)

Additional Talking Points:

- MVP researchers have conducted some of the largest studies to date in diverse populations in areas including: anxiety, depression, PTSD, Agent Orange exposure and lymphoid cancers, corneal dystrophy, suicidal behavior, suicidal ideation, Alzheimer's Disease in people of African descent; and coronary artery disease.

HVAC Subcommittee on Health – Talking Points

MILLION VETERAN PROGRAM

Highlights/Examples of MVP research

- **Largest Study Ever on Genetics and Health:** Through a partnership with the U.S. Department of Energy (DOE), MVP conducted a groundbreaking genetic study involving 350 billion analyses on a supercomputer that resulted in a wealth of resources for future scientific discovery. Researchers investigated links between ~44 million genetic variants across 2,000 health and identified 38,270 independent genetic variants associated with one or more health traits, a third of which are unique to non-European populations. The results are universally available for further research through the National Institutes of Health (NIH) National Library of Medicine.

The study has the potential to identify new uses for FDA-approved drugs, allowing known medications to treat new diseases and health conditions. MVP researchers have mapped more than 4,000 genetic signals to approved drugs. Possible health conditions for repurposed medications include: Lipid-lowering drugs for glaucoma and chronic kidney disease, Antibiotics for digestive disorders and Antidiabetics to prevent atrial fibrillation, a common heart disease.

- **Suicide prevention:** Food insecurity and marital status were shown to increase risk of suicide identified by MVP research. These factors have been incorporated into VA's suicide risk-prediction tool, REACHVET 2.0, which was deployed in the VA in June 2025.
- **Ending Homelessness:** MVP researchers in collaboration the Department of Energy applied artificial intelligence to develop a tool that can predict housing instability in Veterans 2 months in advance of the VA Homeless Screener survey response. Efforts are currently underway to validate these findings within the VA.
- **Early prediction of prostate cancer risk:** A genetic risk score that can help predict risk for developing prostate cancer identified by MVP research is now being tested in a VA clinical trial that is comparing standard of care prostate cancer screening to standard of care plus the genetic risk score.
- **Preventing end-stage kidney disease:** A protective gene for end-stage kidney disease discovered by MVP is currently in a Pharma-sponsored clinical trial.
- **New treatments for Heart Failure through Industry partnerships** –research collaborations between MVP and two industry partners are underway to leverage MVP data for biomarker discovery, validation and drug development and repurposing in heart failure.



TAB 13

Precision Oncology

HVAC Subcommittee on Health – Talking Points

PRECISION ONCOLOGY

BLUF:

- Precision Oncology (PO) efforts are accelerating translation of discoveries to address the real-world needs of Veterans
- PO is maximizing the use of VA data and analytics to foster innovation, fill gaps in knowledge, drive improvements in clinical and research outcomes, and develop new evidence for clinical care
- PO supports an enterprise-wide clinical research infrastructure to leverage expertise, resources, knowledge, bring trials closer to Veterans and foster strong research-clinical collaboration which is essential for achieving real-world impact in Veteran healthcare
- PO as an exemplar of a learning health care model that integrates the clinical domains with research and enable partnerships with VA program and clinical operations offices, federal agencies, industry and non-profits

Statistics:

- Over 50,000 Veterans are diagnosed with cancer annually
- Prostate, lung, colon, bladder are among the highest other than skin cancer
- 12,000 Veterans with metastatic cancer have been genetically screened with somatic next generation testing ; over 1000 Veterans screened and 600 enrolled into interventional VA prostate cancer trials
- Over 108,000 Veterans access precision oncology care via our genitourinary network (POPCaP/GU)
- Curated data on 11 million Veterans to study the burden of prostate cancer and tools to study toxic exposure
- 860K lung cancer screening scans on 388K Veterans completed representing 150% increase since 2021 - (192K scans in FY25 and increasing); achieve 90% adherence to lung cancer screening which is effective for early detection
- Largest lung cancer screening study opened with targeted enrollment of 23,520 Veterans to improve LCS criteria (Nichole Tanner/Renda Weiner -Leads; Charleston & Boston)
-

Budget:

FY25 budget was approximately \$44 million

Additional Talking Points:

- Clinical-research infrastructure (POPCaP, LPOP, GU sites, etc):
 - facilitated an increased number of Veterans obtaining genetic testing, enabling access to appropriately matched therapies

HVAC Subcommittee on Health – Talking Points

PRECISION ONCOLOGY

- Bring trials closer to Veterans via teleoncology, and inter facility consults provide access to subspecialties
- Helps to increase access to care while reducing inequities and disparities in cancer care
- Facilitate cancer screening and early detection – lung, ovarian and breast
Example: Partner with NCI to enroll 24,000 patients in the VANGUARD study - multi-cancer early detection blood-based screening
- Harnessing VA data to improve clinical outcomes:
 - Clinical trial demonstrates that it is possible to treat metastatic prostate cancer without castration for better quality of life in Veterans
 - Development of innovative clinical trials better suited for Veterans and their cancer characteristics – prostate, lung, bladder, pancreas and heme malignancies
e.g., STARPORT – aim to define new standard of care for Veterans with oligometastatic prostate cancer, over 305 Veterans enrolled of 374
RESTORE - Challenges long-standing paradigms of the need for ongoing castration therapy, to improve efficacy of therapy for Veterans with metastatic prostate cancer while simultaneously maintaining or improving quality of life.
 - Investigations into mutational signatures associated with military exposure to improve diagnosis and treatment
 - Research to improve lung cancer and prostate cancer screening criteria and standards
Examples: **Modeling-Informed Prostate Cancer Screening for High-Risk Veterans** - Precision screening using of VA clinical and genomic data to do modeling, trial emulation, causal inference to inform USTPF PSA screening recommendations.

ProGRESS: Prostate cancer, Genetic Risk, and Equitable Screening Study a large clinical trial (5000 Veterans) of precision prostate screening among a diverse group of male Veterans nationwide. Study already enrolled over 2,016 Veterans

MAS Expand - Aim to improve and expand lung cancer screening criteria by incorporating toxic military exposure, genetics, family lung cancer history and history of tobacco related cancers.

HVAC Subcommittee on Health – Talking Points

PRECISION ONCOLOGY

- Coordinated gold standard analyses of VA real-world data that yield high quality results more efficiently that address knowledge gaps, inform clinical pathways, evidence-based guidelines, clinical trials and process improvements
Examples – a. Compare docetaxel vs cabazitaxel use in advanced prostate cancer after one drug already failed
b. Compare how well magnetic resonance imaging (MRI) works in monitoring patients with early-stage cancer vs taking tumor biopsy
- **Leveraging AI/machine learning**, imaging and genomic data to develop tools to stratify patients for treatment and assist in clinician decision making:
 - Lung texture tool to characterize incidental interstitial abnormalities identified on lung cancer scans
 - Automated lung super classifier that predicts potential for a lung nodule to become malignant
 - A radiomic model that predicts therapy-induced) pneumonitis in lung cancer patients receiving chemoradiation and immunotherapy
 - Use of AI/ML to develop super classifier to predict metastatic progression in high risk localized prostate cancer and oropharyngeal cancer; enable VA clinicians to optimize/intensify treatment when appropriate.
- **Meeting the requirements of the Cleland-Dole Act –**
 - Identified 13 Clinical Cancer Research hubs in 13 VISNs, working to install 5 more hubs in FY26/27
- **VA is a key partner in interagency collaborations**
 - ***Applied Proteogenomic Organizational Learning and Outcomes (APOLLO)***
– a DoD, NCI, VA collaboration on the contribution of RNA, proteins and DNA to advance the individualization of cancer care from diagnosis to prognosis and treatments.
 - VA is a major contributor, having enrolled over 1,200 Veterans and in FY25 expanded from 7 vanguard sites (supported by DoD) to 15 VA sites covering 14 VISNs.
 - ***National Cancer Institute Collaborations***
 - National Cancer Institute (NCI) and VA Interagency Group to Accelerate Trials Enrollment (NAVIGATE) – provide NCI supported trials across 16 VA medical centers
 - Open over 657 NCI trials (or 154 unique trials) in NAVIGATE and non-NAVIGATE sites (Lung, GU, GI, Heme, Breast) and enrolled over 1,294 Veterans into trials
 - Large minority enrollment -over 24% African Americans, 68% White

HVAC Subcommittee on Health – Talking Points

PRECISION ONCOLOGY

- ***Cancer Screening Research Network (CSRN)*** - Network to assess emerging approaches to cancer screening (e.g., multi-cancer early detection blood-based screening)
 - VA is serving as an Accrual, Enrollment, and Screening Site (ACCESS) Hub and will participate in accruing 24,000 patients in the VANGUARD trial.
- ***Food and Drug Administration*** – working on a collaboration to explore novel clinical trial designs, dose optimization, implementing novel drug development tools, real-world comparative effectiveness, toxicity and patterns and barriers to new interventional therapy uptake.



TAB 14

Prosthetics

HVAC Subcommittee on Health – Talking Points

PROSTHETICS

BLUF:

- To help meet the lifestyle and medical needs of Veterans who have lost limbs, VA researchers develop and test a wide variety of prosthetic devices.
- VA's goal is to offer Veterans prosthetics that will restore them to their highest possible level of functioning within their families, communities, and workplaces.
- VA Researchers are working on:
 - developing high-functioning artificial limbs that are very similar to their natural counterparts
 - advanced wheelchair designs that promote mobility and independence for wheelchair users and make it easier to use a wheelchair
 - using functional electrical stimulation and other technologies to help those with weak or paralyzed muscles, and developing and testing state-of-the-art adaptive devices to help those with vision or hearing loss.

Statistics:

Budget:

Additional Talking Points:

- AMPDECIDE & PROPREDICT- Shared decision making support tools for VA patients and providers
 - VA investigators at the Puget Sound VA have developed decision support tools to better inform clinicians and patients about their personalized risks and benefits at several amputation levels facilitating a shared decision making process to assist in making decisions that align with each patients values and priorities (<https://www.ampdecide.org>), as well as to better inform clinicians and patients about their probable level of prosthetic mobility assisting in prosthetic prescription options and rehabilitation planning (<https://prodecide.org>).
 - These tools, which are freely available worldwide, were created based on Veteran data and interviews and VHA is now integrating them into the VA Electronic Health Record to make them more available, automated, and easier to use to improve Veteran amputation and prosthetics-related outcomes and satisfaction.

HVAC Subcommittee on Health – Talking Points

PROSTHETICS

- Glean™ Wireless and Catheter-Free Urodynamics System
 - This VA-developed technology recently received FDA clearance and is now available for clinical use, substantially reducing patient discomfort and improving urodynamic testing.
 - Licensed to and available from Bright Uro, a disabled Veteran-owned small business- <http://gleanuds.com/>



TAB 15

Psychedelics

Psychedelic Research at the VA*

VA Offices of Research and Development and Mental Health work closely together to position VA to conduct psychedelic research and plan for potential future clinical implementation of psychedelic treatments for mental health conditions.

VA researchers conduct psychedelic research funded by external sources and VA is beginning to fund research on psychedelics through its intramural research program.

Legal Status of Psychedelics

- ❖ No psychedelics are currently approved by the FDA.
- ❖ Several psychedelics have “breakthrough therapy” status from FDA (MDMA, psilocybin, LSD).
- ❖ Clinical use of psychedelic treatments beyond research will only begin once they are approved.
- ❖ Research is conducted under strict safety protocols and in compliance with Federal guidelines.

Current Clinical Trials with Psychedelics for Mental Health Conditions

 **16**
Studies

23 M \$
External Funding

Location of Psychedelic Trials



Upcoming VA-Funded Clinical Trials

- ❖ MDMA-Assisted Therapy for Veterans with PTSD and Alcohol Use Disorder
- ❖ Multi-Site Trial of Psilocybin for Treatment-Resistant Depression
- ❖ Psilocybin-Assisted Acceptance and Commitment Therapy In Depressive Disorders

* As of November 2025

HVAC Subcommittee on Health – Talking Points

PSYCHEDELICS

BLUF:

- VA researchers are exploring the therapeutic potential of psychedelic compounds, like MDMA and psilocybin for the treatment of mental health conditions.
- As of October 2025, there are 16 clinical research studies on psychedelic treatments for mental health conditions at 10 VA medical centers, focusing on PTSD, Major Depressive Disorder (MDD), generalized anxiety disorder, and substance use disorders. Compounds being studied include:
 - MDMA
 - psilocybin
 - DMT
 - LSD
- Three clinical trials have been selected for funding through VA's intramural research program:
 - MDMA-Assisted Therapy for Veterans with PTSD and Alcohol Use Disorder: A Randomized Controlled Trial (Providence)
 - A Multi-site Randomized Controlled Trial of Psilocybin for Treatment-Resistant Depression (TRD) in Veterans (*Birmingham*, Portland (OR), Puget Sound, Tuscaloosa, Philadelphia)
 - Safety, efficacy, and mechanisms of psilocybin-assisted Acceptance and Commitment Therapy (PACT) in treating Major Depressive Disorder (MDD) among Veterans (*San Francisco*, San Diego, Minneapolis, Palo Alto, Salt Lake City)

Statistics:

Budget:

- ORD expects to spend at least \$7M on the 3 trials described above.
- Based on current activities, it is estimated that non-VA funding sources will provide over \$23 million to VA researchers for psychedelic research.

Additional Talking Points:

- In recent years, scientific evidence for the effectiveness of psychedelic treatments for mental health conditions has been building. It is critical to validate findings in the Veteran population.

HVAC Subcommittee on Health – Talking Points

PSYCHEDELICS

- At this time psychedelics are classified as Schedule 1 substances with no currently acceptable medical use and high potential for abuse. As such, they are not approved for clinical use and VA is unable to either provide or pay for these services outside of research settings.
- ORD and VA's Office of Mental Health (OMH) have been working closely to position VA to conduct psychedelic research in the Veteran population and for the potential future clinical implementation of psychedelic treatments.
- ORD & OMH efforts were formalized by creation of a Psychedelic Medicine Integrated Project Team (IPT) to develop a comprehensive implementation plan for the future clinical deployment of safe and effective psychedelic treatments for mental health conditions in VHA. The IPT consists of five (5) workstreams: 1) Research, 2) Clinical Implementation, 3) Training, 4) Informatics & Evaluation and 5) Community Care.
- As with all VA clinical research, psychedelic treatments are conducted in a clinical setting with strict safety protocols and in compliance with all appropriate Federal guidelines for conducting studies with controlled substances.
- The table below provides a list of Active Clinical Trials on Psychedelic Treatment for Mental Health Conditions

Project Title	Location(s)	Psychedelic	MH Condition
A Dose Optimization Study of MDMA-Assisted Therapy for U.S. Veterans with PTSD	Bronx, NY	MDMA	PTSD
Persistence of Effectiveness, Functional and Health Outcomes, VA Service Utilization and Biomarkers following MDMA-Assisted Therapy for PTSD in Veterans	Bronx, NY	MDMA	PTSD
Open-label Phase 2 study of MDMA-Assisted Psychotherapy in Veterans with Combat-Related, Refractory PTSD.	Loma Linda, CA	MDMA	PTSD
A Randomized, Double-Blind, Single-Site Phase II 2-Arm Study To Compare the Safety and Preliminary Efficacy of Manualized MDMA-Assisted Therapy to Low Dose D-Amphetamine Assisted Therapy in Veterans For The Treatment of Moderate to Severe PTSD	Los Angeles, CA	MDMA	PTSD
A randomized trial to compare MDMA-assisted therapy (MDMA-AT) versus Cognitive Processing Therapy (CPT), a VA standard-of-care psychotherapy for PTSD, for the Treatment of Severe Posttraumatic Stress Disorder	Palo Alto, CA	MDMA	PTSD

HVAC Subcommittee on Health – Talking Points

PSYCHEDELICS

Psilocybin-Enhanced Psychotherapy for Methamphetamine Use Disorder	Portland, OR	Psilocybin	Substance Use
A Phase 3, Randomized, Double-Blind, Multicenter Study to Evaluate the Efficacy, Safety, and Tolerability of Psilocybin in Adults with Major Depressive Disorder	Portland, OR Los Angeles, CA Omaha, NE Bronx, NY	Psilocybin	Depression
A Randomized, Double-Blind, Placebo-Controlled, Clinical Trial of MDMA-Assisted Therapy for Active Duty Service Members with PTSD	San Diego, CA	MDMA	PTSD
Investigating the neural mechanisms underlying the effects of MDMA combined with prolonged exposure therapy on PTSD in veterans	San Diego, CA	MDMA	PTSD
MDMA-Assisted Brief Cognitive Behavioral Conjoint Therapy for PTSD	San Diego, CA	MDMA	PTSD
MDMA-Assisted Massed Prolonged Exposure for PTSD	San Diego, CA White River Junction, VT	MDMA	PTSD
A Phase 3 Trial of MM120 for Generalized Anxiety Disorder (VOYAGE)	San Francisco, CA	LSD	Generalized Anxiety Disorder
The potential therapeutic effects of psychedelic, N, N-dimethyltryptamine (DMT), on alcohol use disorder (AUD)	West Haven, CT	DMT	Substance Use
Fixed Order, Open-Label, Dose-Escalation Study of DMT in Humans	West Haven, CT	DMT	Depression
Phase 1 Study of the Safety, Tolerability, Electrophysiological Effects and Efficacy of DMT in Humans	West Haven, CT	DMT	Depression
Psilocybin-Induced Neuroplasticity in the Treatment of Major Depressive Disorder	West Haven, CT	Psilocybin	Depression



TAB 16

Research Workforce (Field)

HVAC Subcommittee on Health – Talking Points

RESEARCH WORKFORCE ISSUES

BLUF:

- The research workforce is complex and composed of VA compensated employees (Research, Medical Care, Medical Care Support, and Office of Information Technology), without compensation employees and Intergovernmental Personnel Act detailed personnel.
- Most Personnel are on not to exceed time limited positions tied to projects.
- Positions can range from clinical (physicians, nurses, psychologists) to research specific series (biologists, statisticians, data scientists, health science specialists) to administrative (committee managers, regulatory and compliance)

Statistics:

- Approximately 20,000 positions, mostly on time-limited/project-linked appointments, located across the VAMCs are engaged in research activities. Of this figure, nearly 3,700 are currently funded through the ORD appropriation on Not-to-Exceed (NTE) appointments. An additional 760 positions are permanent/career VA appointments (<4%).
- Additionally, approximately 1328 FTE received protected effort from the medical care appropriation in support of research activities. Most of these are physicians conducting research.

Additional Talking Points:

VA Research has many parts but is structurally comprised of two key components: the Office of Research and Development (ORD) in VA Central Office that establishes policies, requirements, and manages research appropriations; and the field research programs maintained at over 100 VA medical centers (VAMCs), who further report to their respective Veterans Integrated Service Network (VISN).

VAMC priorities may consider clinical and scientific needs and strengths, the Veteran population served, and/or relationships with nearby academic affiliates (i.e., university/medical school). Some VAMCs, VISNs, and VHA Program Offices outside of ORD may also financially support additional research activities and provide personnel. For example, the Office of Mental Health supports centers such as the National Center for Posttraumatic Stress Disorder and Mental Illness Research, Education and Clinical Centers (MIRECCs). Congress established Geriatric Research, Education and Clinical Centers (GRECCs) that are supported by the Office of Geriatrics and Extended Care and Parkinson's Disease Research, Education and Clinical Centers. In cases like these, program offices collaborate with ORD in areas of shared interests.

HVAC Subcommittee on Health – Talking Points

RESEARCH WORKFORCE ISSUES

Salary support may also come from external sources, including grants and other awards from the National Institutes of Health, Department of Defense, industry, or non-profit entities.

Field personnel are managed locally and under the ultimate responsibility of the VAMC Director and his/her Chief of Staff, R&D Office and its Associate Chief of Staff – R&D.

Of the roughly 3,700 field-based positions currently supported by ORD funding, most are temporary positions with NTE appointments -- normally coinciding with the project time period or not to exceed three years, whichever is less. This approach is critical in budget and position management, allowing local research offices to efficiently manage their budget allocation while maintaining adequate staffing levels to support the ongoing projects. The few permanent positions supported by ORD are mostly administrative or regulatory compliance positions in local research offices and represent less than 5% of overall staffing.



TAB 17

Rural Health

HVAC Subcommittee on Health – Talking Points

RURAL HEALTH

BLUE:

- From 2006 to 2025, 40 VA research projects were completed with a total budget of \$34.9 million.
- These projects emphasize improving healthcare access for rural Veterans by leveraging telehealth and virtual care models to provide specialized care in remote areas.
 - Topics include mental health services, women's health, chronic disease management, and the integration of primary and specialty care.
- VA has 2 Centers of Innovation (COINs) totaling \$1.1M in FY25 with a specific impact goal focused on rural health, and 1 Consortia of Research (CORE) center (\$441K FY25) focused on access and community care to support and inform national priorities for achieving high-quality and timely access to care.
- Between Fiscal Year 2020 and FY2025, QUERI invested over \$5 million in projects focused on evaluating and improving the quality of rural Veteran health in areas such as telehealth, lung cancer screening, pain treatment, and cardiovascular care.
 - In addition, between FY20 and FY25, QUERI provided approximately \$61.5 million to support 14 large centers focused on implementing evidence-based practices (EBPs) for Veteran health, such as age-friendly health systems, mental health care and suicide prevention, and substance use disorder treatment.
 - QUERI also supports evaluation of Section 401 of the MISSION Act, which involves developing criteria for designating VA medical facilities as underserved—particularly in rural areas—and guiding resource targeting under Section 402.

Statistics:

- There are 13 active projects in the rural health research portfolio totaling \$10.2 million. The projects focus on diverse themes, including suicide prevention, telehealth, pain management, increasing access to care, 1 on women's health, traumatic brain injury care, cardiac rehabilitation, lung cancer screening, and on mental health. Of these 13 projects, 4 are clinical trials described below.
 - Improving dermatology access by direct-to-patient teledermatology and computer-assisted diagnosis.
 - Reducing Chronic Opioid Use Among Veterans Undergoing Community Care Surgery Using a Transitional Pain Service.
 - Advancing Decisions about Virtual Service Encounters (ADVISE).
 - Linda G. Park: Virtual Care Strategies to Improve Participation in Cardiac Rehabilitation among Veterans
- As of FY2026, QUERI is actively supporting 8 rural health-focused projects with a total planned investment of over \$7.6 million through FY2030. These projects aim to improve access, quality, and equity of care for rural Veterans through

HVAC Subcommittee on Health – Talking Points

RURAL HEALTH

innovative virtual care models, targeted evaluations, and national program support.

Budget:

- \$10,218,896.50 is allocated for 13 active projects.
- \$34,946,565.00 was spent on 40 research projects related to rural health since 2006.
- \$1.1M in FY25 was spent on 2 Centers of Innovation (COINs) that have a specific impact goal focused on rural health.
- \$441K in FY25 was spent on 1 Consortia of Research (CORE) center focused on access and community care.
- >\$5,000,000 in QUERI funds were allocated to rural health evaluation projects between FY20 and FY25. QUERI plans to invest \$7,600,000 in rural health projects through FY 30.

Impact:

- Digital health-electronic tablets improved mental health care access for over 222,000 Veterans (35% rural)
 - Veterans who received tablets experienced increased mental health service use and 36% reduction in the likelihood of having a suicide-related emergency department visit ([Zulman 2022](#))
- Veterans Crisis Line (VCL) Caring Letters initiative associated with improved mental health care access
- PTSD Coach mobile app improved access and quality, outcomes, and satisfaction for Veterans with PTSD
- VA researchers found a 63% increase in dual MA-VHA enrollment from 2011 to 2020, accounting for \$78 billion in VHA-spending for potentially duplicate services.

QUERI Active Projects as of 10/1/2025

- Virtual Care QUERI 1.1

Supporting the national rollout of three equity-focused, evidence-based practices via virtual platforms: tele-collaborative mental health care, computerized cognitive behavioral therapy, and pain self-management tools for rural Veterans.

- Carriage III QUERI (VAST EBP)

Implementing and evaluating a multidisciplinary video-based antimicrobial stewardship team to improve prescribing practices in rural VA medical centers.

- Evaluation of the National Telenephrology Hub and Spoke Network

HVAC Subcommittee on Health – Talking Points

RURAL HEALTH

Assessing the impact of ORH's telenephrology initiative on Veteran reliance on VA nephrology services, Community Care spending, and wait times.

- Evaluation of Tele-Hospital Medicine

Evaluating the economic impact and value of the enterprise-wide tele-hospital medicine initiative in rural settings.

- Enhancing Veterans' Access to Care through Video Telehealth Tablets

Evaluating the implementation and effectiveness of VA's tablet distribution initiative on rural Veterans' access to care and clinical outcomes.

- Center for the Evaluation of Enterprise-Wide Initiatives (CEEWI)

Supporting the VA Office of Rural Health by evaluating the implementation, sustainment, and spread of national programs that improve the health and well-being of rural Veterans.

- eHealth Partnered Evaluation Initiative

Supporting the Office of Connected Care's mission to further implementation of VA connected care technologies and to assess their impact on VA healthcare delivery and outcomes.

- Mobile Lung Screening Evaluation and Outcomes

Evaluating the implementation of mobile lung cancer screening in 19 community-based outpatient clinics in VISN 7 and 9.

Additional Talking Points:

- Population: ~4.4 - 4.7 million Veterans live in rural communities (»25% of U.S. Veterans).
- VA Enrollment: ~2.7 - 2.8 million rural/higher-rural Veterans are enrolled in VA health care (»60%+). Over half (»54%) are age 65 or older.
- Health Needs: Rural Veterans are generally older and carry a higher burden of service-related conditions compared to urban Veterans.
- Mental Health: Suicide risk is higher in rural Veterans (~20–22% higher than urban). Past VA data show suicide rates ~33.3 vs 29.1 per 100,000 among rural vs urban VA users.
- **MISSION Act** Creation of underserved model for primary care to help target resources to underserved facilities (Sections 401-402): top five underserved facilities serve a disproportionate number of rural Veterans; >40% of Veterans at the top five facilities live in rural areas, traveling an average of about 18 miles to the nearest VA facility.

HVAC Subcommittee on Health – Talking Points

RURAL HEALTH

- Access Barriers: Rural Veterans face long travel distances to VA facilities, fewer local providers, and transportation challenges.
- Telehealth & ORH Impact: In FY2023, ~685,020 rural Veterans received ORH-supported telehealth services. ORH invested ~\$96M in FY2024 for clinical resource hubs and expanded telehealth in primary, mental health, specialty, and home-based care.
 - Sources: VA Office of Rural Health, VA Suicide Prevention Program, VA Office of Connected Care (2024 data), National Center for Veterans Analysis and Statistics (2023–2024 data).



TAB 18

Sensitive Species

HVAC Subcommittee on Health – Talking Points

SENSITIVE SPECIES – ANIMAL RESEARCH

BLUE:

- Section 247(g) of the FY24 appropriations acts requires that VA eliminate research in sensitive species (cats, dogs, and nonhuman primates) not later than March 9, 2026.
*****New CR extended deadline to September 20, 2026**
- VA is complying with these Congressional requirements and moving toward the elimination of research involving sensitive species.
- VA currently has five active studies in sensitive species that were previously permissible; all five studies will be closed prior to March 2026.
- VA would need legislative changes – superseding the FY24 Act requirements – to enable the commencement of new research involving sensitive species after March 9, 2026.

Statistics: The number of VA studies involving sensitive species has declined drastically, by more than 93%, over the last twenty years. Nonhuman primates used in VA research have gone from over 800 to 9, dogs from 700 to 5, and cats from 400 to 0.

Budget: VA is not forecasting any budgetary allocations for sensitive species research after December 2025.

Additional Talking Points:

In compliance with the Further Consolidated Appropriations Act 2020, the VHA Office of Research and Development (ORD) developed a “Five Year Plan to Eliminate or Reduce Research Conducted Using Canine, Felines, or Nonhuman Primates” to “reduce or eliminate” sensitive species research and submitted this plan for Congressional approval. Specifically, ORD has cataloged, characterized, and appropriately scoped sensitive species research, explored alternative approaches, and conducted sensitive species research commencing on or after October 1, 2019 under permitted Secretary waivers.

The Consolidated Appropriations Act, 2024 stated “The Department shall implement a plan under which the Secretary **will eliminate the research** conducted using canines, felines, or non-human primates by not later than 2 years after the date of enactment of this Act.”

This is different language than prior Congressional direction and OGC concluded that this language requires the elimination of VA sensitive species research by March 2026, including any active studies at that time. OGC has determined that the law does not permit any waivers; thus, VA would need legislative changes – superseding the FY24 Act requirements – to enable the commencement of new research involving sensitive species after March 9, 2026.

HVAC Subcommittee on Health – Talking Points

SENSITIVE SPECIES – ANIMAL RESEARCH

Currently approved protocols in sensitive species:

- Canine
 - Mechanistic Insight of Premature Ventricular Contractions-induced Cardiomyopathy
 - Autonomic Nerve Activity and Cardiac Arrhythmias
 - Administration of Intratumoral Immunocytokine to Activate Immune Rejection of Spontaneous Canine Melanoma
- NHP
 - Stem Cell Therapy for Treatment of Spinal Cord Injury
 - Contusion Injury as a Model for Spinal Cord Injury

The 5 dogs referenced in the statistics section were pets, volunteered by their owners, that naturally developed melanoma and who would die without intervention.



TAB 19

Suicide Prevention and PTSD

HVAC Subcommittee on Health – Talking Points

SUICIDE PREVENTION AND PTSD

BLUF:

- Suicide prevention continues to be a research (and clinical) priority for VA.
 - Priorities:
 - Firearms and Lethal Means
 - Transitioning Service Members
 - Altitude and Suicide
 - Building Infrastructure to Support Researchers
- PTSD research continues to be a priority for VA.
- Innovative Treatments
 - Examples of ongoing VA Clinical Programs for Veterans based on VA Research-funded Suicide Prevention Research:
 - Supporting Patients after Discharge from the Emergency Department (SPED)
 - Standardized Suicide Risk Screening and Evaluation (VA Risk ID)
 - REACH VET (Recovery Engagement and Coordination for Health – Veterans Enhanced Treatment)
 - FINVET (National Veterans Financial Resource Center)
 - VA DoD Clinical Practice Guidelines
 - Caring Contacts
 -
- Continued collaboration with DoD on Study to Address Risk & Resilience in Servicemembers (STARRS)
 - Longitudinal study to understand suicide risk & mental health.
 - Established Researcher-in-Residence Program to allow VA investigators to partner w/ STARRS for full access to this massive longitudinal database.

Statistics:

118 ORD-funded Suicide Prevention Projects are active

8 active projects study interaction of suicide risk and PTSD.

- Budget:
 - Approximately \$20M for all related projects in FY26
 - Approximately \$1.9 million for projects specific to suicide and PTSD.

HVAC Subcommittee on Health – Talking Points

SUICIDE PREVENTION AND PTSD

Research impacts:

Examples of ongoing VA Clinical Programs for Veterans based on VA Research-funded Suicide Prevention Research:

- Supporting Patients after Discharge from the Emergency Department (SPED)
- Standardized Suicide Risk Screening and Evaluation (VA Risk ID)
- REACH VET (Recovery Engagement and Coordination for Health – Veterans Enhanced Treatment)
- FINVET (National Veterans Financial Resource Center)
- VA DoD Clinical Practice Guidelines
- Caring Contacts
- Veteran Sponsorship Initiative

Additional Talking Points:

ORD intends to support a Suicide Prevention Precision Health Research Center.

The Center would be tasked with curating VA data to allow for phenotyping of suicide risk and develop research projects will allow for development of precision treatment rules.

The Research center will commence October 1, 2027. The center is designed to address the issue that current treatments are applied uniformly and not tailored to the individual's risk. The aim of the center is to create precision treatment rules that will improve treatment effectiveness and enhance efficiencies for the organization.

Additionally, to augment the efficient development of suicide prevention interventions, the Suicide Prevention AMP is including a common data element requirement for all funded projects. Common data elements will be stored in a central repository and linked to electronic health records allowing secondary analysis of studies across VA. This initiative is in response to recommendations from the VA/DoD Suicide Prevention Clinical Practice Guidelines.



TAB 20

TBI and Blast Injuries

HVAC Subcommittee on Health – Talking Points

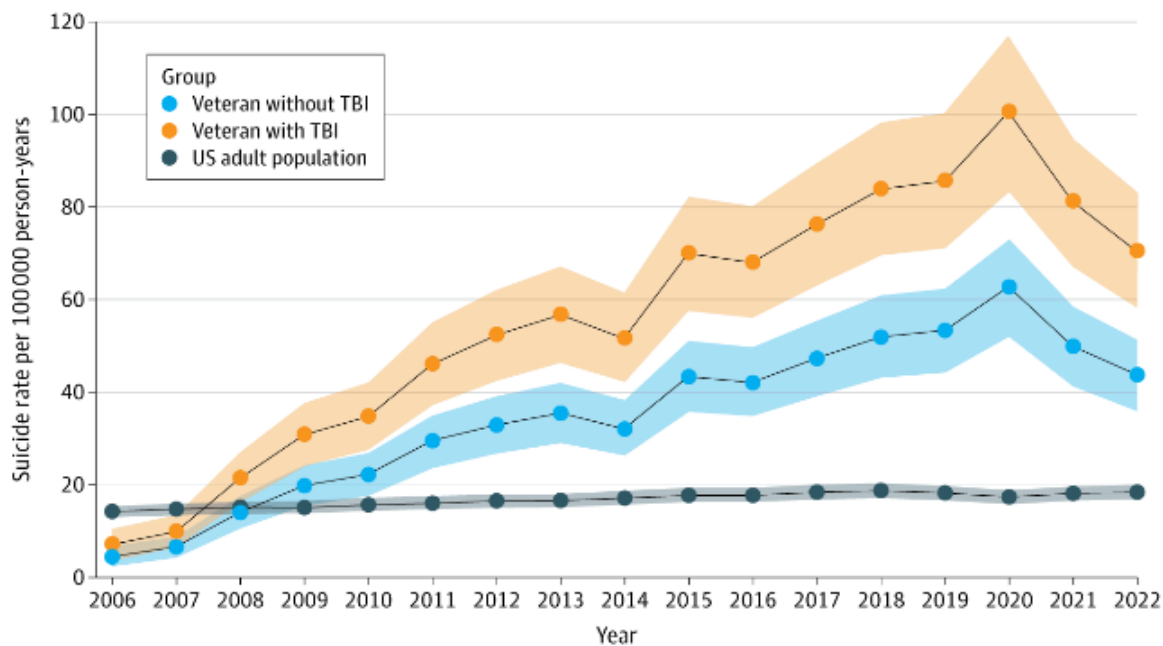
TBI and BLAST INJURIES

BLUF:

- Traumatic brain Injury (TBI) is known as a signature injury of the Post 9/11 conflicts.
- High-level blast exposure from improvised explosive devices (IEDs) resulted in loss or alteration of consciousness in those in proximity to these blasts and were subsequently diagnosed with TBI.
- The long-term effects of having one or more TBIs is currently being studied in both prospectively followed cohorts of Veterans and through epidemiological studies.
 - Preliminary findings indicate that Veterans with these injuries are at higher risk for mental health conditions, cardiovascular disease, and various sensory deficits.

Statistics:

- Over 225,000 Veterans utilizing VHA for health care have a history of at least one TBI diagnosis.
- Over a third of those who screened positive for a possible service-connected TBI (156,317) have not returned for a comprehensive TBI exam to confirm.



Budget: \$44 million projected for FY2026 across all portfolios, \$35 million for TBI AMP alone.

HVAC Subcommittee on Health – Talking Points

TBI and BLAST INJURIES

Additional Talking Points:

Since the beginning of FY2024, the VA has been investigating whether low-level blast exposure, specifically those exposures that occur in high frequency from routine training or operation of specific weapon systems, can also impact brain and mental health. The concern with low-level blast is that these exposures rarely cause an alteration or loss of consciousness that would trigger the diagnosis of TBI. Veterans who had these types of military occupational blast exposures are at greater risk for wide range of mental and physical health problems, including, but limited to suicide, all cause mortality, bipolar disorder.

VA investigators have identified distinct patterns, or "phenotypes," of persistent health problems and long-term outcomes for those who have sustained a TBI and/or Military Occupational Blast Exposure (MOBE)

Data:

We are capitalizing on trove of longitudinal data from LIMBIC-CENC that includes biomarkers. These data are available to other researchers through FITBIR

- FITBIR – Federal Interagency Brain Injury Research Informatics System is a centralized informatics system (database) for TBI and Brain Health research housed and supported by National Institutes of Health. In addition, it receives support from DoD and VA.
 - It serves as a central repository for new data, links to current databases and allows valid comparison of results across studies.
 - The database builds upon an effort to create common data elements for the study of TBI and related Brain Health conditions- which are essentially definitions and guidelines about the kinds of data that should be collected, and how to collect these data in clinical studies.
 - Common data elements are key for combining data from different studies



TAB 21

Veterans Engagement

HVAC Subcommittee on Health – Talking Points

VETERAN ENGAGEMENT

BLUF:

- Veteran Engagement Groups (VEGs) are commonly used by VA researchers to ensure that Veterans have a voice in shaping the research that is for and about them.
- Within ORD, the HSR and Rehabilitation Research, Development and Translation broad portfolios utilize VEGs.
- ORD's Million Veteran Program and Cooperative Studies Program also engage Veterans in the research process but do so with mechanisms other than standing VEGs.

Statistics:

Budget:

Additional Talking Points:

BLUF: This briefing describes standing Veteran engagement groups (VEGs) that support Health Systems Research (HSR) for VA's Office of Research and Development (ORD). Findings underscore the importance of Veteran engagement groups for advancing Veteran-centric research and for positively and directly impacting Veterans.

Background: VEGs are commonly used by VA researchers to ensure that Veterans have a voice in shaping the research that is for and about them. Within ORD, the HSR and Rehabilitation Research, Development and Translation broad portfolios utilize VEGs. ORD's Million Veteran Program and Cooperative Studies Program also engage Veterans in the research process but do so with mechanisms other than standing VEGs.

We sought to describe HSR VEGs, characterizing their composition, mechanics, activities, and impacts.

Methods: A brief survey was distributed to VEG facilitators across VA HSR Centers of Innovation (COINs; n=20) and Consortia of Research (COREs; n=7) in May 2025. For this assessment, VEGs associated with individual research projects were not included. The survey consisted of closed-ended questions to assess VEG composition and mechanics as well as open-ended questions to assess VEG activities and impacts. We tabulated response selections for closed-ended questions and used qualitative content analysis to analyze open-ended questions.

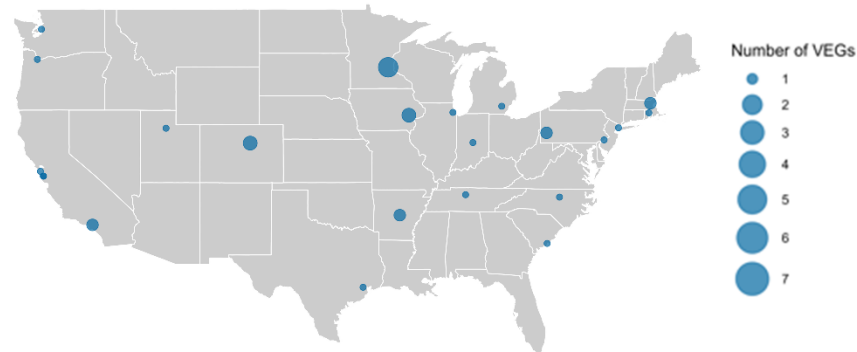
Characteristics of VEGs in HSR: Facilitators from all VEGs associated with COINs and COREs responded, providing information on 37 unique VEGs. Figure 1A and Table 1 denote all VEGs included in the analyses, and their locations. About half of the VEGs (n=21) reported a specialty focus (e.g., Veterans experiencing homelessness, Veterans diagnosed with Long COVID) while the remainder were more general in what research topics they discussed. VEGs ranged in size (3-17 members, median: 9); eleven included Veteran care partners in addition to Veterans and a small subset also included VA employees or others (Figure 1B). Most groups (n=30) met virtually at a variety of cadences, ranging from twice per year to every other week (Figure 1C).

Bidirectional impacts between VA research and Veterans: VEGs confer several benefits to investigators and other parties (e.g., VA's national clinical/policy program offices) as well as to Veterans. A prominent benefit is that Veterans in VEGs provide input to research along the full research lifecycle—including advising on research priorities; reviewing project proposals; providing feedback on study design, methods, and results; and disseminating and promoting research—with the goal of making research more Veteran-centric. (Figure 2 provides a summary of all identified parts of the research lifecycle.) VEGs also engage in social gatherings and strategic planning for their VEG and some VEG members serve as participants in research studies and/or are members of research project teams.

VEG facilitators relayed that Veterans reported experiencing many positive impacts from participating in VEGs. These included having the chance to make a difference and have their voices heard, gaining increased trust and understanding of VA research and clinical care, experiencing personal development and content learning (e.g., about health topics), and having opportunities to participate in high-profile research activities outside of the VEG. Veterans also relayed that there were social and personal health benefits of participating in VEGs.

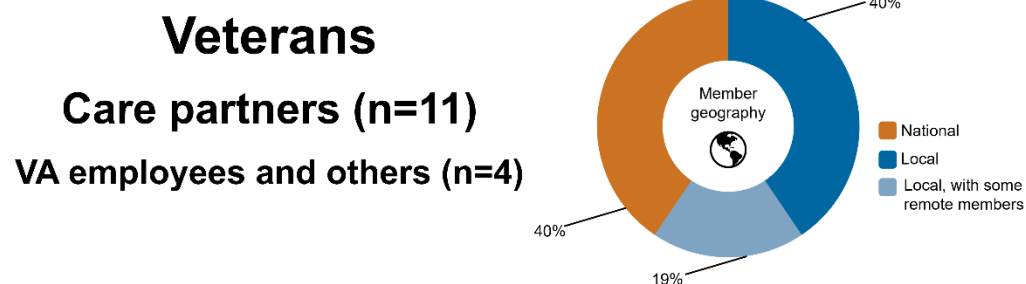
Where are HSR Veteran engagement groups?

A.



Who comes to Veteran engagement groups?

B.



What are meetings like?

C.

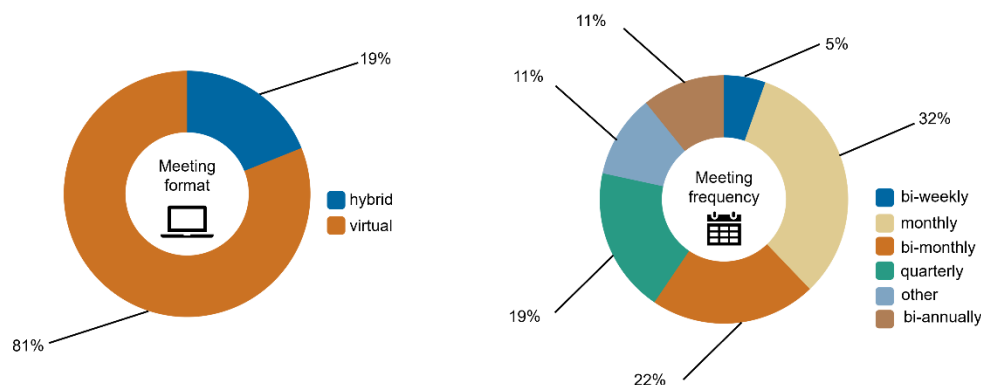


Figure 1. (A). Locations of Veteran engagement groups (VEGs) in HSR. Dot size indicates number of groups in a particular geographic area. Donut plots of VEG characteristics for n=37 groups: member geography (B), meeting format (C - left), meeting frequency (C - right). All VEGs had Veteran members.



Figure 2. Veterans provide input to investigators and other groups along the full spectrum of the research lifecycle.

Table 1. Alphabetized list of all Veteran engagement groups (VEGs) that completed the assessment. VEGs are affiliated with a HSR Center of Innovation (COIN) or Consortia of Research (CORE). All COINs and COREs responded to the data call.

3DI Veteran Engagement Group (VEG)	Substance Addiction & Recovery Veteran Engagement Board (SAR-VEB)
ACCENT Veteran and Caregiver Engagement Panel	VA COIN Veterans Advisory Board
CeMHOR Veteran Engagement Council (VEC)	VA Pittsburgh Healthcare System Veteran Advisory Board
Center for Health Information and Communication (CHIC) Veteran and Community Engagement Group (VCEG)	Veteran and Family Engagement Group
CHOIR Veteran Engagement in Research Group (VERG)	Veteran Economic Security Panel (VESP)
CINCCCH Veteran Engagement Panel	Veteran Engagement Group (VEG) at the Center to Improve Veteran Involvement in Care (CIVIC), Portland, Oregon
Community Engagement Committee	Veteran Impact Panel on Research (VIP-R)
Greater Los Angeles (GLA) Veteran Engagement Team (VET)	Veteran Research Engagement Board (VREB), Charleston, SC
IDEAS COIN Veteran Research Engagement Panel (VETREP)	Veteran Research Engagement Board (VREB), Aurora, CO
Iowa City CADRE-ORH Veteran Engagement Panel	Veteran Research Engagement Council (VREC)
Long COVID Veteran Engagement Panel	Veterans and Family Research Engagement Council (VFREC)
Lung Precision and Oncology Program (LPOP)	Veterans Community Advisory Board (VCAB)
Overdose Prevention Veteran Engagement Panel (VEP)	Veterans Research Engagement Group (VetREP)
Pain/Opioid Veteran Engagement Panel (VEP)	VETWISE-LHS Veteran Engagement Panel
Providence COIN VEG	Virtual Care CORE Veteran Engagement Panel
Rural Colorado Veteran Research Engagement Board (RCVREB)	Voices of Healing
Rural Midwest Veteran Panel	Women's Health Panel, Minneapolis, MN
Seattle Veteran Engagement Group for Research	Women's Improvement Network (WIN)
SPRINT Veteran Engagement Council	



TAB 22

Visual Health

HVAC Subcommittee on Health – Talking Points

VISUAL HEALTH

NOTE: Information on Iowa City eye research is at the end of this summary.

BLUF:

- The Rehabilitation Research Development and Translation (RRDT) Broad Portfolio supports two vision research centers (see below).
 - VA Center for the Prevention and Treatment of Visual Loss, Iowa City, Iowa
 - Center for Visual and Neurocognitive Rehabilitation, Atlanta GA.
- Researchers at the Cleveland VA discovered an enzyme that is a potential biomarker for diagnosing and treating diabetic retinopathy, damage to the blood vessels in the eye that can lead to blindness.
- An ophthalmologist at the San Francisco VA invented a device to help Veterans administer eye drops with the goal of assisting Veterans who have less strength and dexterity in their hands.
- Researchers with the VA Million Veteran Program identified distinct genetic signals for age-related macular degeneration, a disease that causes vision loss, across different ancestries. The study shows the importance of including diverse study populations to uncover ancestry-dependent risk factors and potential therapeutic targets.

Statistics: Veterans with visual dysfunction receiving compensation: 431,776

Budget: ~\$6M in FY25 for the Centers and individual projects

Additional Talking Points:

ORD funds two Centers with a focus on visual and neurocognitive rehabilitation:

1. VA Center for the Prevention and Treatment of Visual Loss, Iowa City, Iowa

Mission/Goals: To provide new discoveries for restoring vision and related brain functions, in order to improve veteran quality of life. Disorders that are targeted for rehabilitation include glaucoma, diabetic retinopathy and neuropathy, macular degeneration, TBI, optic neuropathy, multiple sclerosis, raised intracranial pressure, light sensitivity, and migraine.

Focus Area 1: Rehabilitation of Vision Loss: New treatments for diabetic retinopathy and neuropathy, glaucoma, optic nerve disorders, macular degeneration, CNS and systemic vascular disorders.

HVAC Subcommittee on Health – Talking Points

VISUAL HEALTH

Focus Area 2: Ocular Biomarkers for Rehabilitation of CNS Disease. Examples of CNS disorders with quantifiable ocular biomarkers include migraine, photosensitivity, double vision, stroke, pain, impulsivity/suicide and dementia.

Focus Area 3: Automated Telemedical Assessment of Ocular Disorders and Assistive technologies for the Visually Impaired. Examples: Smartphone testing of visual function remotely, development of auditory and vibratory input to enhance navigation.

Examples of Studies in Vision:

1. Evaluating the interaction of the immune system and inflammation on the progression of blast-mediated neurodegeneration (PI: Harper: 3/2021-2/2025)
2. Omega-3 Polyunsaturated Fatty Acids in the Treatment of Diabetic Peripheral Neuropathy: Is the Source Important? (PI: Yorek: 7/2021-6/2025)
3. Exploring the implications of widespread decreases in dynamic blood flow to the retina and choroid in dry macular degeneration (PI: Linton: 5/2024-4/2026)
4. Translational Studies Toward Cell Therapy for Treatment of Primary Open Angle Glaucoma (PI: Kuehn: 9/2024-8/2028)
5. Early learning detection of progressive visual loss in glaucoma using deep learning (PI: Garvin: 6/2022-5/2026)
6. Development of Automated Motility and Strabismus Evaluation for diagnosis and treatment (PI: Dumitrescu: 1/2025-12/2029)
7. Investigation of thalamocortical circuits for the treatment of headache (PI: Sowers, 6/2022-5/2026)

2. Center for Visual and Neurocognitive Rehabilitation, Atlanta GA.

The mission of the Atlanta VA CVNR is to develop novel, clinically relevant rehabilitative approaches based on scientific discoveries that improve the health and quality of life of Veterans with visual or neurocognitive deficits.

Examples of VA ORD funded studies include:

1. Treating Early-Stage Diabetic Retinopathy (PI: Pardue: 6/1/2022-5/31/2026)
2. The Relationship of Menopause to the Onset of Glaucoma in Veterans (PI: Feola: 11/1/2023-9/30/2026)
3. Impact of Menopause on the Aqueous Outflow Pathway (PI: Feola: 9/1/2021-7/31/2025)
4. The Active AMD Study To Improve Function in Veterans with Age Related Macular Degeneration (PI: Nocera: 10/1/2023-9/30/2027).

HVAC Subcommittee on Health – Talking Points

VISUAL HEALTH

Neurocognitive: Studies Not related to Visual Function:

1. Intention Treatment for Anomia: Investigating Dose Frequency Effects and Predictors of Treatment Response to Improve Efficacy and Clinical Translation
2. rTMS in Alleviating Pain and Co-morbid Symptoms in Gulf War Veterans (PI: Rauch: 10/1/2019-9/30/2025)
3. Partnered Dance Aerobic Exercise as a Neuroprotective, Motor and Cognitive Intervention in Parkinson's (PI: Hackney: 11/1/2019 -9/30/2024)
4. Intention Treatment for Anomia: Investigating Dose Frequency Effects and Predictors of Treatment Response to Improve Efficacy and Clinical Translation (
5. Innovative Memantine Therapy for Neuroprotective Effects Against Ischemic Stroke and Alzheimer's Disease (PI: Yu: 11/1/2022-10/31/2024)

IOWA City Eye Research

Excerpt from testimony from Dr. Mahsaw Mansoor, Resident Physician, Iowa City VA:

Currently, there are multiple AI research projects at the Iowa City VA Center for the Prevention and Treatment of Blindness (CPTVL). We are extremely grateful to have support and funding from the VA Rehabilitation Research and Development Division. This team is led by Dr. Randy Kardon, who is an internationally recognized expert in neuro-ophthalmology and an extremely accomplished vision researcher. Grant funding was recently renewed for our fourth 5-year cycle starting in July of this year. This includes core funding from the Rehabilitation and Development Division of \$1.2 million per year. There is additional funding directly from the National Institutes of Health and Department of Defense that increases our research funding to about \$7 million per year currently. We are grateful for this support as it has been able to catalyze AI research at the Iowa City VA.

The research team led by Dr. Kardon focuses on the application of AI and deep learning to quantify and visualize the spatial patterns of nerve loss in the retina in highly impactful blinding diseases affecting Veterans and citizens with glaucoma, age-related macular degeneration, DM, multiple sclerosis, ischemic optic neuropathy, radiation retinopathy, and eye movement disorders.

About Digital Diagnostics – An Iowa Founded AI Company

Digital Diagnostics is a healthcare technology company founded by Dr. Michael Abramoff in Coralville, Iowa. As a healthcare technology company, Digital Diagnostics designs and implements AI systems that can diagnose disease by analyzing high-quality images. Digital Diagnostics' mission is to benefit patients by developing technology to make healthcare

HVAC Subcommittee on Health – Talking Points

VISUAL HEALTH

easier to access, affordable, available to everyone, and of the highest quality. Built on a foundation of bioethical principles, Digital Diagnostics is creating impactful AI diagnostic systems that improve patient outcomes and address existing barriers to care. Using AI, we can create solutions that help increase access, reduce costs, and improve the quality of care for those that need it most.

Comments from John Kaplan, Technology Transfer:

- Based on the testimonial record, Dr. Mansoor refers to Digital Diagnostics. According to our information in working with the University of Iowa, Digital Diagnostics has an exclusive license for a number of inventions involving Dr. Michael Abramoff. Recently, University of Iowa (UIRF) put in place an updated license that restates and bundles all Digital Diagnostics' IP into one single agreement.
- VA asserted rights on several of these inventions and has received royalty payments related to the license.
- <close hold> Dr. Abramoff reached out to Tech Transfer on/around 11/4 and mentioned something in his email about University of Iowa doing a due diligence process... it sounds like an acquisition may be occurring, but we are not sure—he did not answer that question when I asked him via email.



TAB 23

Women's Health

WOMENS HEALTH

BLUF:

Amputation, prosthetics, and rehabilitation

- Meeting the needs of women Veterans who have lost limbs is not simply a case of miniaturizing solutions that work for men.
- Include studies that span needs, preferences and functional abilities with upper and lower limb loss, digit loss, and more, as well as functional demands of Veteran mothers.

Aging and Women Veterans

- Evaluate the needs of midlife women Veterans and inform clinical trials development and strategic policy planning to support the needs of perimenopausal and menopausal women Veterans.
- Revisit clinical studies for hormone replacement treatments for menopause.
- An [ongoing pre-clinical study](#) is rethinking the prescribing of bone-building, antiresorptive bone treatments, and probiotics in early menopause on bone quality and fracture resistance in post-menopause.

Precision Oncology and Women's Health

- Work with the National Precision Oncology Program to build the Breast and Gynecologic Oncology System of Excellence, including a Center for Women's Cancer Research.
- Study the relationship between military toxic exposures and cancer risk, including breast cancer.

Precision Health and Women's Health

- Enhance recruitment/enrollment of Veterans from underrepresented populations such as women for the Million Veteran Program.
- Increase the enrollment of Activity Duty Women Service Members and Women Veterans in CSP 505: Millennium Cohort Studies
- Enhance enrollment of Women in the Veterans' Affairs Rheumatoid Arthritis Registry to address treatment gaps for autoimmune conditions.

HVAC Subcommittee on Health – Talking Points

WOMENS HEALTH

Rural Health and Women's Health

- Side-by-side with clinical counterparts in VHA, an ORD study through the Quality Enhancement Research Initiative (QUERI) program is expanding access to women-centric telehealth services focused on diabetes prevention, CVD risk, and perinatal depression prevention. (EMPOWER initiative).

Training and Mentoring in Women's Health Research:

- Department of Veterans Affairs (VA) and National Institutes of Health (NIH) will offer early-career VA researchers an opportunity to participate in NIH's Office of Research on Women's Health Building Interdisciplinary Research Careers in Women's Health (BIRCWH) program.

Mental Health and Women's Health

- An ORD-funded study (detailed below) found that one out of four women Veterans struggle with depression, and pregnant Veterans have an increased risk of postpartum depression compared to their civilian counterparts.
- One in three women Veterans suffer from intimate partner violence (Iverson et al. 2002) IPV is associated with mental health issues such as low self-esteem, quality of life, depression, PTSD and suicidality.
- Understand the factors that affect suicide risk among women Veterans, especially those [not using VA healthcare](#).

Statistics:

Women Veterans are the fastest growing group of the Veteran population. Between fiscal years (FYs) 2000 and 2023, the total percentage of women Veterans increased from 6.3% to 11.3% or 2.1 million of the total Veteran population in the United States. By 2043, 2.2 million women are projected to make up 17.2% of all living Veterans.

- The median age of women Veterans was 52 in 2023. Approximately 22.5% were 65 or older, and 2.2% were 85 or older.
- In 2023, 35.7% of women Veterans resided in five states, Texas, Florida, California, Virginia, and Georgia. By 2043, 36.6% of women Veterans are projected to reside in Texas, Florida, California, Georgia, and North Carolina

HVAC Subcommittee on Health – Talking Points

WOMENS HEALTH

Source: <https://www.data.va.gov/stories/s/wci3-yrsv>

Health Utilization among Women and Men Veteran Patients (FY20):

- The top three domains for women overall were Musculoskeletal, Mental Health/SUD, and Endocrine/Metabolic/Nutritional and for men overall were Endocrine/Metabolic/Nutritional, Cardiovascular and Musculoskeletal.
- In patients 18-44 years old, Mental Health/SUD and Musculoskeletal were the most frequent domains for both women and men (Mental Health/SUD: 57.4% vs. 50.3%); Musculoskeletal: 50.4% vs. 48.7%).
- In women 45-64 years old, Musculoskeletal was the most common domain and this domain was higher ranked and more frequent in women than in men (60.8% vs. 54.5%).
- In patients 65+ years old, the top two domains for women and men were Endocrine/Metabolic/Nutritional and Cardiovascular (Endocrine/Metabolic/Nutritional: 64.7% vs. 62.3%; Cardiovascular: 58.2% vs. 64.1%).

<https://www.womenshealth.va.gov/materials-and-resources/publications-and-reports.asp#sourcebook-v5>

Budget:

The total congressional appropriation for VA medical and prosthetic research was \$916 million in fiscal year 2023. Approximately \$40 million of that amount was used to support VA research on health issues that affect Women Veterans across their lifespan.

<https://www.research.va.gov/currents/1124-White-House-Executive-Order-on-Womens-Health-Research.cfm>

Additional Talking Points:

Amputation, prosthetics, and rehabilitation

A new prosthetic device for shoes with different heel heights.

Historically, prosthetic feet have not worked well with shoes of different heel heights, such as flats, boots, stilettos, and sneakers. Researchers recently developed a prosthetic ankle-

HVAC Subcommittee on Health – Talking Points

WOMENS HEALTH

feet system that allows people with amputations to wear nearly any pair of shoes. As part of the initial testing, three women with leg amputations who were regular prosthesis users tested out the new system by walking around in three pairs of shoes with varying heel heights that they wanted to wear.

Publication: [Functional dynamic prosthesis alignment maintained across varying footwear using a modular prosthetic ankle-feet system](#) (Walker et al., 2025, *PLOS One*).

Aging and Women Veterans

Menopause among women Veterans

Researchers are fielding a Veteran-centered needs assessment in 30 VA Medical Centers about menopause. The assessment will evaluate the needs of midlife women Veterans and inform clinical trials development and strategic policy planning to support the needs of perimenopausal and menopausal women Veterans.

Precision Oncology and Women's Health

Coordinating access to mammograms for VA patients

VA patients are often referred outside of VA medical facilities (known as Community Care) for mammograms. In this study, 44 VA staff members across 10 VA facilities were interviewed about the process of coordinating Community Care mammograms. Staff noted that there was variability in community provider processes and that not all community providers were familiar with VA processes.

Publication: [“We Are Working Harder, Not Smarter”: A Qualitative Inquiry into Care Coordination for Department of Veterans Affairs Mammograms Referred to the Community](#) (Monty et al., 2023, *Women's Health Research*).

Military exposures and cancer in women Veterans

A new study (still in JIT) will look at the relationship between toxic exposures and cancer risk, including breast cancer, among military firefighters. The study will include a cohort of 5,783 women Veterans.

WOMENS HEALTH

Mental Health and Women's Health

One out of four women Veterans struggle with depression, and pregnant Veterans have an increased risk of postpartum depression compared to their civilian counterparts. Over the past four years, VA has supported implementation of the ROSE (Reach Out Stay Strong Essentials) evidence-based intervention to prevent perinatal depression; hundreds of VA-based staff (e.g., nurses, social workers) have been trained to deliver ROSE. Veterans who have participated in ROSE have shared its impact on their communication with their partners. This work is related to the [Enhancing Mental and Physical Health of Women through Engagement and Retention \(EMPOWER\) 2.0 QUERI Program](#).

One in three women Veterans suffer from intimate partner violence. IPV is associated with physical and mental health issues such as low self-esteem, quality of life, depression, PTSD, suicidality and homelessness (Iverson et al. 2002; Iverson et al. 2003). Research awards funded by VA ORD resulted in implementation of IPV screening protocol, training of providers, and testing of an individualized psychosocial counseling intervention program called Intimate Partner Violence through Strengths and Empowerment (RISE).

<https://pubmed.ncbi.nlm.nih.gov/34813687/>; <https://www.psychiatrist.com/jcp/rct-intervention-for-intimate-partner-violence-veterans/>

Pain:

Migraines are more common in Women Veterans

A large study of more than 491,000 Veterans (450,625 men and 40,979 women) looked at the relationship between migraines, other health conditions, and environmental exposures during military service. Women Veterans reported a significantly higher rate of migraines: 30.1% vs. 8.2%. Veterans with a history of migraines also reported worse overall health, higher likelihood of psychiatric conditions, higher levels of pain, and greater lifetime opioid use.

Publication: [Migraine prevalence, environmental risk, and comorbidities in men and women Veterans](#) (Gasperi et al., 2024, JAMA Network Open).

Suicide:

A need for suicide prevention strategies geared toward Women Veterans

HVAC Subcommittee on Health – Talking Points

WOMENS HEALTH

This study was the first longitudinal, national survey of Veterans with a non-fatal suicide attempt, conducted to better understand the differences between male and female Veterans in their psychosocial symptoms and suicidal thoughts and behaviors.

Publication: [Gender differences in risk and resilience for suicidal thoughts and behaviors: A national longitudinal survey study of United States veterans with a recent suicide attempt](#) (Denneson et al., 2024, *Journal of Affective Disorders*).

Office of Research and Development (ORD) activities:

- In 2010, ORD invested in the [Women's Health Research Network \(WHRN\)](#) to systematically transform VA's capacity to examine and reduce gender disparities in health care and use research to increase delivery of evidence-based care tailored to Women Veterans.
- In 2024, ORD established a Women's Health Research Integration Workgroup (WH RIG) to accelerate women Veterans' research with content expertise from WHRN and VA National Program Offices, e.g., Office of Women's Health (OWH) and Office of Mental Health and Suicide Prevention (OMHSP).
- The WH RIG activities included developing the first [Women Veterans' Health and Healthcare Notice of Special Interest \(NOSI\)](#), updating reviewer criteria and guidance associated with the Inclusion of Women section in RFAs, and dissemination of information about [VA's Women's Enhancement Recruitment Process](#) (WERP) Toolkit to improve recruitment of women Veterans to participate in research.
- The [Women Veterans' Health and Healthcare Notice of Special Interest \(NOSI\)](#) focuses on Women Veterans' access to healthcare and conditions that affect women Veterans uniquely (e.g., breast and gynecological cancers, menopause, endometriosis, uterine fibroids, fertility, reproductive health, etc.), disproportionately (e.g., autoimmune diseases, arthritis, chronic kidney disease, etc.), differently (e.g., cardiovascular disease, etc.), and conditions related to military service (e.g., military exposures) as well as congressionally directed priority areas including prosthetic and other assistive technology needs of women Veterans.

HVAC Subcommittee on Health – Talking Points

WOMENS HEALTH

Interagency Activities in Women's Health:

- Department of Veterans Affairs (VA) and National Institutes of Health (NIH) will offer early-career VA researchers an opportunity to participate in NIH's Office of Research on Women's Health Building Interdisciplinary Research Careers in Women's Health (BIRCWH) program
https://www.research.va.gov/programs/womens_health/bircwh.cfm
- The Department of Veterans Affairs (VA) and the Department of Defense (DoD) have established a Women's Health Research Collaborative to identify specific actions and key milestones to advance women's health research for Women Service Members and Veterans. Near term goal includes increasing awareness of VA and DoD's research activities with a longer-term goal of convening a research priority setting conference.
<https://news.usuhs.edu/2024/10/usu-celebrates-military-womens-health.html>
- National Academy of Sciences Engineering and Medicine Forum on Regenerative Medicine hosted a workshop entitled: [The Intersection of Regenerative Medicine and Womens Health A Workshop | National Academies](#) on Oct 1st, 2024 which was co-chaired by representatives from VA ORD and NIH ORWH. The proceedings of the workshop: *The Intersection of Regenerative Medicine and Women's Health*, was released in April 2025. (<https://nap.nationalacademies.org/read/29088>), and meeting comments were captured on their interactive webpage (<https://nap.nationalacademies.org/resource/29088/interactive/>). The output of this workshop has been adopted as part of the Strategic Plan for the NIH/Office of Research on Women's Health.